


# Leadership Roles and Management Functions in Nursing

**Assist. Prof. Dr. Hala Aiash**

**BSN, MPH, PHD**

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# Chapter one

# Decision Making, Problem Solving, and Critical Thinking Requisites for Successful Leadership and Management

***. . . the successful nurse executive has the ability to make good decisions consistently.***

**—THOMAS R. CLANCY**

***. . . in any moment of decision, the best thing you can do is the right thing, the next best thing is the wrong thing, and the worst thing you can do is nothing.***

**—THEODORE ROOSEVELT**

This chapter explores the primary requisites for successful management and leadership: decision making, problem solving, and critical thinking.

Decision making, is both an innermost leadership activity and the core of management.

# 1.DECISION MAKING

Because managers spend much time making decisions and solving problems, developing skills in these areas is essential to increasing effectiveness.

Nurse manager is confronted by a variety of situations. Hospital or agency policies provide guidelines for dealing with routine situations. But exceptional instance can make decisions more difficult and require a mature sense of judgment.

## 1.1 DECISION MAKING

**Decision making** is a complex, cognitive process often defined as choosing a particular course of action.

Defines decision making as "the process of making choices or reaching conclusions

Choosing from alternatives a particular course of action. To arrive at a solution that ends uncertainty.

Is a choice made between two or more available alternative? Both definitions imply that: uncertainty---choosing one alternative.

## 1.2 PROBLEM SOLVING

Problem Solving is a systematic process leading to the achievement of outcomes. The ability to solve problems effectively contributes to the delivery of safe, competent nursing care. Problem solving always involves a decision making.

Decision making, on the other hand, is usually triggered by a problem but is often handled in a manner that does not focus on eliminating the underlying problem.

For example, if a person decided to handle a conflict crisis when it occurred but did not attempt to identify the real problem causing the conflict, only decision-making skills would be used.

The decision maker might later choose to address the real cause of the conflict or might decide to do nothing at all about the problem. The decision has been made *not* to problem solve.

This alternative may be selected because of a lack of energy, time, or resources to solve the real problem. In some situations, this is an appropriate decision.

## 1.4FACTORS INFLUENCING DECISION MAKING

### 1. Decision makers attribute

- Knowledge, experience and judgment
- Perception and personality
- Values and philosophy

### 2.The Situation

- Urgency of solution and time pressures
- Magnitude and importance
- Structure and uncertainty and risk
- Cost benefit

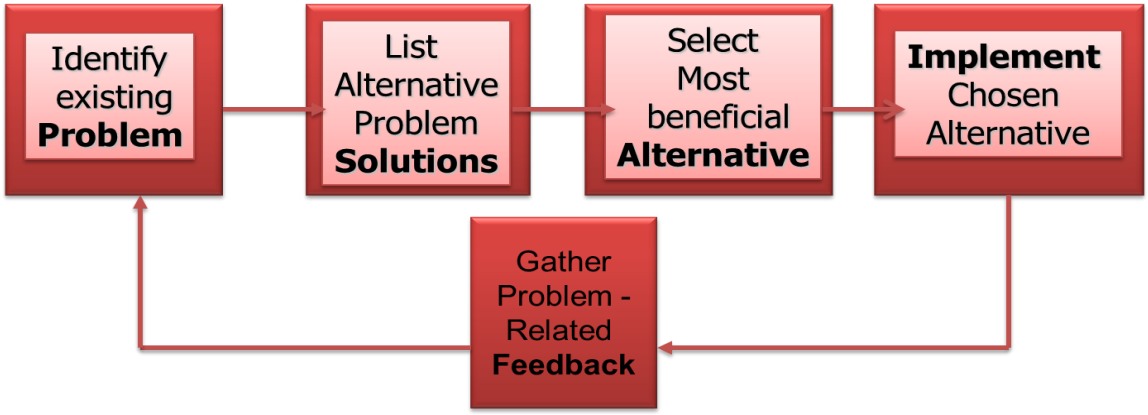
### 3. Environmental Constraints

- External
- Internal

## 1.5 THEORETICAL APPROACHES TO PROBLEM SOLVING AND DECISION MAKING

1) Traditional Problem-Solving model
2) Managerial Decision Making
3) Nursing Process

## 1.5. THE DECISION-MAKING PROCESS




### 1.5.1 -THE TRADITIONAL PROBLEM-SOLVING MODEL

Widely used and the most well-known model

Includes 7 steps:

1- Identify the problem.

2- Gather data to analyze (causes of problem).

3- Explore alternative solutions.

4- Evaluate the alternatives.

5- Select the appropriate solution.

6- Implement the solution.

7- Evaluate the results.

### 1.5.2-THE MANAGERIAL DECISION-MAKING MODEL

Includes 6 steps:

1- Set objectives.

2- Search for alternatives.

3- Evaluate alternatives.

4- Choose.

5- Implement.

6- Follow up and control.

### 1.5.3- THE NURSING PROCESS

Includes 4 steps:

1- Assessment.

2-Planning.

3- Implementation.

4- Evaluation.

### 1.5.4 The IDEALS Model

The *IDEALS model*, as it came to be known, includes six steps for effective thinking and problem solving.

- **Identify the problem.** "What's the real question we're facing here?"
- **Define the context**. "What are the facts and circumstances that frame this problem?"
- **Enumerate choices**. "What are our most plausible three or four options?"
- **Analyze options**. "What is our best course of action, all things considered?"
- **List reasons explicitly**. "Let's be clear: Why are we making this particular choice?"
- **Self-correct.** "Okay, let's look at it again. What did we miss?"

## 1.6 CRITICAL THINKING

Critical Thinking: "the mental process of actively and skillfully conceptualizing, applying, analyzing, synthesizing, and evaluating information to reach an answer or conclusion"

Critical thinking is reflecting upon the meaning of statements, examining the offered evidence and reasoning, and forming judgments about facts

## 1.7 THE DIFFERENCE BETWEEN THINKING AND CRITICAL THINKING

Critical thinking is controlled, purposeful & more likely to lead to obvious beneficial results

Thinking is basically any mental activity; can be aimless & uncontrolled; it may serve a purpose, but we often aren't aware of its benefits; we might not even remember our thoughts at all

## 1.8 ARE CRITICAL THINKING AND PROBLEM SOLVING THE SAME?

**NO!** Decision making, one step in the problem-solving process

They are sometimes used interchangeably but are separate processes that are related in some situations.

Not all decisions we make involve solving a problem; creativity is a form of critical thinking itself.

## 1.9 CRITICAL ELEMENTS IN PROBLEM SOLVING AND DECISION MAKING

**1. Clearly defined objectives**, decision makers often go ahead in their problem-solving process; without first determining their goal which result in poor quality decisions.

**2. Careful data gathering** one must learn how to obtain accurate information and avoid bias.

**3. Generate many alternatives**:

- Do not limit your choice to tow possible judgments when there are more options available.

- Increasing the number of people working on a problem will increase the number of alternatives that can be generated.

Brain storming: thinks of all possible alternatives even those that may seem off target.

**4. Think logically:**

- Draw inference from information.

- Careful thinking through the information and alternatives.

- Faulty logic leads to poor quality decisions.

**5. Choose and act decisively:**

Many people become vulnerable at this point in problem _solving process and choose to delay acting because they lack the courage to do so

Good decision _makers seem to have some sort of antenna that makes them particularly sensitive to situations than others.

**1.10 The following to be qualities of a successful decision maker:**

**1- Courage.**

Courage is particularly important and involves the willingness to take risks.

**2- Sensitivity**.

Good decision makers seem to have some sort of antenna that makes them particularly sensitive to situations and others.

**3- Energy.**

People must have the energy and desire to make things happen.

**4- Creativity.**

Successful decision makers tend to be creative thinkers. They develop new ways to solve problems

## 1.11 SUMMARY

This chapter has discussed effective decision making, problem solving, and critical thinking as requisites for being a successful leader and manager.

The effective leader–manager is aware of the need for sensitivity in decision making.

The successful decision maker possesses courage, energy, and creativity.

It is a leadership skill to recognize the appropriate people to include in decision making and to use a suitable theoretical model for the decision situation.

# Chapter 2

## Leadership and Management

***. . . management is efficiency in climbing the ladder of success; leadership determines whether the ladder is leaning against the right wall.***

**—STEPHEN R. COVEY**

***. . . no executive has ever suffered because his subordinates were strong and effective.***

**—PETER DRUCKER**

This chapter will first artificially differentiate between management and leadership, focusing on how theory development in each field of study has changed over time and then conclude with a discussion of how closely integrated the two roles must actually be for individuals in contemporary leadership or management roles.

Managers are essential to any organization. A manager's functions are vital, com- plex, and frequently difficult. They must be directed toward balancing the needs of patients, the health care organization, employees, physicians, and self. Nurse managers need a body of knowledge and skills distinctly different from those needed for nursing practice, yet few nurses have the education or training necessary to be managers. Frequently, managers depend on experiences with former supervisors, who also learned supervisory techniques on the job. Often a gap exists between what managers know and what they need to know.

Today, all nurses are managers, not in the formal organizational sense but in practice. They direct the work of nonprofessionals and professionals in order to achieve desired outcomes in patient care. Acquiring the skills to be both a leader and a manager will help the nurse become more effective and successful in any position.

## 2.1 MANAGEMENT

Management is the process of leading and directing all or part of an organization through the deployment and manipulation of resources.

Encarta World English Dictionary (2009, para 1) defines *management* as "the act of handling or controlling something successfully" or "the skillful handling or use of something such as resources."

## 2.2 MANAGERS

A manager, in contrast, is an individual employed by an organization who is responsible and accountable for efficiently accomplishing the goals of the organization.

Managers focus on coordinating and integrating resources, using the functions of planning, organizing, supervising, staffing, evaluating, negotiating, and representing.

## 2.2.1 THE MANAGER'S JOB IS TO:

- Clarify the organizational structure
- Choose the means by which to achieve goals
- Assign and coordinate tasks, developing and motivating as needed
- Evaluate outcomes and provide feedback

## 2.2.2 CHARACTERISTICS OF MANAGERS

1. Have an assigned position within the formal organization
2. Have a legitimate source of power due to the delegated authority that accompanies their position
3. Are expected to carry out specific functions, duties, and responsibilities
4. Emphasize control, decision making, decision analysis, and results
5. Manipulate people, the environment, money, time, and other resources to achieve organizational goals.
6. Have a greater formal responsibility and accountability for rationality and control than leaders
7. Direct willing and unwilling subordinates.

## 2.2.3 MANAGEMENT LEVELS:

**Top level management**: responsible for overall operations ex. (chief officer executive – director of nursing)

**Middle level management**: coordinates activities of several unites (supervisors)

**First line managers**: responsible for actual production of services (head nurse)

## 2.2.4 MANAGEMENT SKILLS:

**Technical skills:** involves method, procedures, techniques and knowledge of work being performed in the organization

**Human Relation skills:** ability to motivate and educate others and to manage conflicts.

**Conceptual skills:** requires analytical thinking and being able to perceive trends and to conceive the organization as a whole.

## 2.3 LEADERS

The word leadership was not known in the English language until the first half of the 19th century.

Ward's (2009, para 1) definition of leadership as "the art of motivating a group of people to act towards achieving a common goal,"

### 2.3.1 LEADERSHIP ROLES

1- Decision maker 2- Coach 3- Forecaster 4- Communicator 5- Counselor 6- Influencer 7- Evaluator 8- Teacher 9- Creative 10- problem solver 11- Facilitator 12- Critical thinker 13- Advocate 14- Mentor Role model

## 2.4 LEADER

leader is the inspiration and director of the action. He or she is the person in the group who possesses the combination of personality and skills that makes others want to follow his or her direction"

The Process of directing the **behavior** of others toward the accomplishment of objectives

Leaders are in the front, moving forward, taking risks, and challenging the status quo

### 2.4.1 CHARACTERISTICS OF LEADERS

1. Leaders often do not have delegated authority but obtain their power through other means, such as influence.
2. Leaders have a wider variety of roles than do managers.
3. Leaders may or may not be part of the formal organization.
4. Leaders focus on group process, information gathering, feedback, and empowering others.
5. Leaders emphasize interpersonal relationships.
6. Leaders direct willing followers.

7. Leaders have goals that may or may not reflect those of the organization.

### 2.4.2 Ten Fatal Leadership Flaws

1. A lack of energy and enthusiasm
2. Acceptance of their own mediocre performance
3. Lack of a clear vision and direction
4. Having poor judgment
5. Not collaborating
6. Not walking the talk
7. Resisting new ideas
8. Not learning from mistakes
9. A lack of interpersonal skills
10. Failing to develop others

**Do we need leaders or managers?**

**All organization need effective leaders and good managers.**

## 2.5 HISTORICAL DEVELOPMENT OF MANAGEMENT THEORY

Management science, like nursing, develops a theory base from many disciplines, such as business, psychology, sociology, and anthropology.

### 2.5. 1-Scientific Management (1900–1930)

**Fredrik Taylor, t**he father of scientific management in early 1990. Scientific management, classical theory, views organization as a machine.

This theory emphasized rapid and efficient production. The primary goal of any organization should be:

- To increase production and efficiency.
- To decrease wasted human efforts.

**The four overriding principles of scientific management as identified by Taylor (1911) are:**

1. By using time and the expertise of experienced workers, work could be scientifically designed to promote greatest efficiency of time and energy.
2. Taylor thought that each employee's abilities and limitations could be identified so that the worker could be best matched to the most appropriate job.
3. Taylor viewed humans as "economic animals" motivated only by money.
4. The relationship between managers and workers should be cooperative and interdependent. The role of managers was to plan, prepare, and supervise. The worker was to do the work

### 2.5.2-Management Functions Identified (1925)

Herni Fayol, first identified the management functions of planning, organizing, staffing, directing, coordinating, reporting & budgeting,

Management process: planning, organizing, staffing, directing & controlling

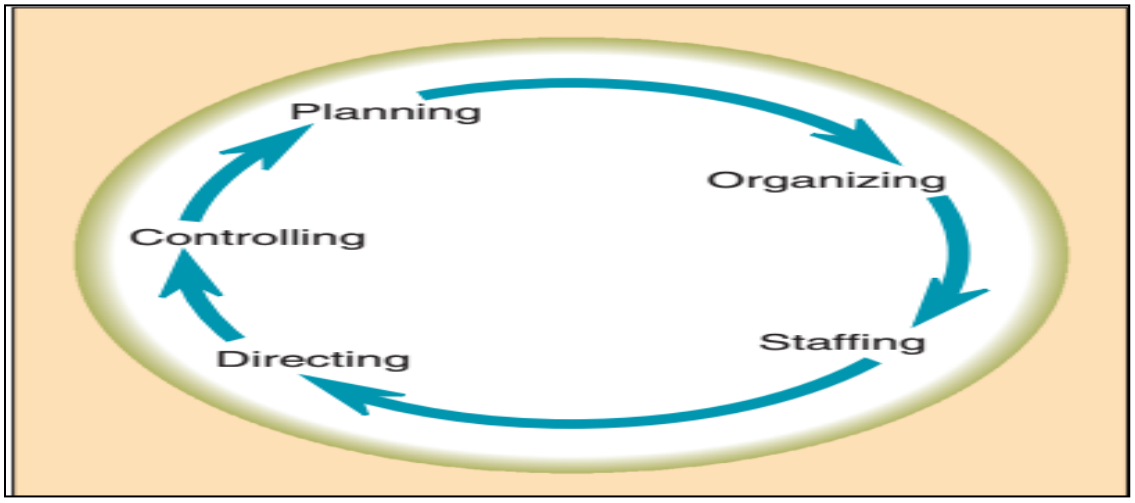



**Figure 2.1 The management process.**

## The Five Functions of The Management Process

### 1. Planning:

Involves determining philosophy goals, objectives, policies, procedures, rules, and fiscal planning & managing changes.

### 2. Organization (organizing):

Includes establishing the structure of organization to carry out the plan, determine the most appropriate type of patient care delivery system & grouping activities to meet unit goals.

### 3. Staffing:

Includes recruiting, interviewing, hiring, orienting staff, scheduling and staff development.

### 4. Directing:

Includes motivating, managing conflict, delegation, communication and facilitating collaboration.

### 5. Controlling:

Includes performance appraisal, fiscal accountability, quality control, professional and collegial control.

### 2.5.3. Human Relations Management (1930–1970)

Developed the concepts of participatory and humanistic management, emphasizing people rather than machine. Satisfaction of employee. Time consuming and often resulted in unmet organizational goals. Not every employee liked working in a less structured environment.

## 2.6. HISTORICAL DEVELOPMENT OF LEADERSHIP THEORY

Like management theory, leadership theory has been dynamic; that is, what is “known” and believed about leadership continues to change over time.

### 2.6.1. The Great Man Theory/Trait Theories (1900–1940)

Focus on the assumption that some people are natural leaders. Based on the inherited personal characteristics of the leader. (leader are born not made) who have certain characteristic or personality traits that make them better leader than others.

Some common traits

1. Later theories suggested that traits could be obtained through learning and experience.
2. Intelligent
3. Initiative
4. Creativity
5. Communication skills
6. Emotional maturity
7. Persuasiveness
8. Ability to participate in social activities

### 2.6.2. Behavioral Theories (1940–1980)

Emphasize on what a leader does (behavior) in achieving organizational goals.

**McGreoger** theories have much influence on leadership and management. He focused what the leader do (leader style) rather than what traits a leader has. These styles, authoritarian, democratic, and laissez-faire.

**1-AUTHORITARIAN STYLE**

**Autocratic (Directive):** manager makes decision and announces it

The **authoritarian** leader is characterized by the following behaviors:

* Strong control is maintained over the work group.
* Others are motivated by coercion.
* Others are directed with commands.
* Communication flows downward.
* Decision making does not involve others.

* Emphasis is on difference in status ("I" and "you").
* Criticism is punitive.

**CAN BE USED:**

- Underqualified subordinates.
- Those who don't have the sense of responsibility.
- Large group with a predetermined performance standard.
- Emergency situations.

**ADVANTAGES**

1. Group action are well determined.
2. Reduce work group frustration.
3. Give members a feeling of security.
4. Productivity is usually high (quantity).
5. It is suitable in time of crises when there is no time for group discussion.

**DISADVANTAGES**

1. Reduce creativity
2. Decrease self –motivation
3. Decrease autonomy.

**2-DEMOCRATIC STYLE**

**Democratic (Participative):** manager defines limits; asks group to take decision**.**

The **democratic** leader exhibits the following behaviors:

* Less control is maintained.
* Economic and ego awards are used to motivate.
* Others are directed through suggestions and guidance.
* Communication flows up and down.

* Decision making involves others.
* Emphasis is on “we” rather than “I” and “you.”
* Criticism is constructive.

Because many people must be consulted, democratic leadership takes more time and, therefore, may be frustrating for those who want decisions made rapidly.

**CAN BE USED:**

1. Creative work.
2. Experienced and qualified subordinates.
3. Who have the sense of responsibility?
4. Decision makers.

**ADVANTAGES**

1. Promote autonomy and growth in individuals.
2. Effective when cooperation and coordination is necessary (team work).
3. Appropriate for groups who work together for long period.

**DISADVANTAGE**

1. May be frustrating for others who want decision made rapidly
2. Less efficient quantitatively than authoritarian style

## 3- LAISSEZ-FAIRE STYLE

**Laissez-faire (Permissive- non directives):** manager permits subordinates to function within limits defined by superior

The **laissez-faire** leader is characterized by the following behaviors:

* Is permissive, with little or no control.
* Motivates by support when requested by the group or individuals.

* Provides little or no direction.
* Uses upward and downward communication between members of the group.
* Disperses decision making throughout the group.
* Places emphasis on the group.
* Does not criticize

A person's leadership style has a great deal of influence on the climate and outcome of the work group

**ADVANTAGES**

1. Appropriate when problems are poorly defined and brainstorming is needed to generate alternative solutions.
2. Much creativity and productivity can result when group members are highly motivated and self –directed.

## The Best Style

It was believed that most leaders had a predominant style and use it consistently, but new theories have demonstrated that individual leaders are capable of dynamically moving along the continuum situation.

### 2.6.3-Situational and Contingency Leadership Theories (1950–1980)

No one set of traits and no one leadership style is effective in all situation. A person may be a leader in one situation and a follower in another. They began to realize that other factors in the work environment influence outcomes as much as leadership styles.

### Situational theories

Based on the situational in term of:

* The organization size and structure

* The climate in the organization either supportive or not
* The leader characteristic (power- influence- authority)
* The follower characteristics (knowledge –skills- maturity)

**Contingency theories**

The leadership style will be effective or ineffective depending on three situational factors:

**1-Leader member relations:** the amount of confidence and loyalty the followers have in their leader.

**2-Task structure:** the extent to which the task is defined and its accomplishment can be measured (every one knows exactly what to do).

**3- Position power:** the authority inherent in a position, the power to reward or punish and the organization support for one decisions.

* Directors (high position= hire or fire)
* Elected committee chairpersons- low position.

**2.6.4-Interactional Leadership Theories (1970–Present)**

**Focus on the main three issues:**

1. Leader's behavior
2. Maturity of the group
3. Effectiveness of the leader

**Schein's** model based upon system theory. **Schein Model, based on system theory has the following assumption:**

* People are very complex and highly variable; they have multiple motives for doing things.

* People's motives do not stay constant but change over time.

* Goals can differ in various situations.

* A person performance and productivity are affected by the nature of the task, by his experience and motivation.

* There is no single leadership strategy that will be effective in every situation.

### 2.6.5-Transactional and Transformational Leadership

Burns (1987): both the leaders and follower have the ability to raise each other to higher level of motivation and morality based on building trust, in self-esteem in one's self and others which creates a shared commitment to excellence and mutual growth.

Make work place meaningful, inspiring and motivational Transactional leaders focus on tasks and getting the work done. Transformational leaders focus on vision and empowerment.

The two different types of leaders is shown in Table 2.2.

| Transactional Leader | Transformational Leader |
|---|---|
| Focuses on management tasks | Identifies common values |
| Is a caretaker | Is committed |
| Uses trade-offs to meet goals | Inspires others with vision |
| Does not identify shared values | Has long-term vision |
| Examines causes | Looks at effects |
| Uses contingency reward | Empowers others |

### 2.7. Leadership & Management

* Leadership & management are intertwined and it is difficult one without the another.

* Leadership is the ability to guide or influence others.
* Management is the coordination of resources (time, people, supplies) to achieve outcomes
* People are led, whereas activities and things are managed.
* Leaders are able to motivate and inspire others.
* Managers have assigned responsibility for accomplishing the goals of an organization
* The relationship between leadership and management continues to prompt debate.
* Leadership is viewed by some as one of management many functions; others.
* Leadership requires more complex skills than management and that management is only one role of leadership.
* Management emphasizes control of hours, costs, salaries, overtime, use of sick leave & supplies – whereas leadership increases productivity by maximizing workforce effectiveness.
* In fact, the experienced professional people often say that most of the major problems conflicts and challenges that they face in their work are not technical problems but people problems.

# Chapter 3

# Roles and Functions in Planning

***. . . in the absence of clearly defined goals, we are forced to concentrate on activity and ultimately become enslaved by it.***

***. . . he who fails to plan, plans to fail.***

## LEARNING OBJECTIVES

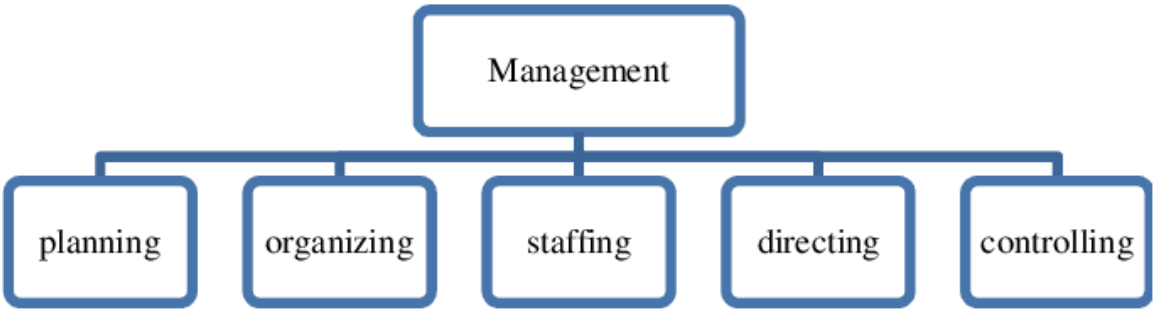
1. Recognize the components of management process

2. Define vision, mission, philosophy, goals, objectives, policy, procedures, rules and regulations

3. Explain the relationship of vision, mission, philosophy, goals, objectives, policy, procedures, rules and regulations.

4. Discuss the importance of mission, philosophy, goals, objectives, policy, procedures, rules and regulations of an organization for nursing service administration

**Management Process Components:**



## 3.1 PLANNING

**Planning**: the organizational process of creating and maintaining a plan; and the psychological process of thinking about the activities required to create a desired future.

Planning is the first a management function, concerned with defining goals for future performance and deciding on the tasks and resources to be used to attain those goals. It is largely conceptual but the results of planning are visible.

The manager needs to be familiar with decision making process and tool so that he/she can identify the purpose of organization, state the philosophy, define goals and objectives policies and procedures, prepare budgets to implement the plans effectively and manage the time.

## 3.2 TYPES OF PLANNING

**1-Strategic planning:** Strategic planning is the "big thinking" concepts. Creating long term plans and goals.

**2-Operational (Tactical) planning:** Tactical planning is the medium- and short-term concepts.

**3-Contingency planning:** Contingency planning. is the "what if?" proactively deal with problems.

### 3.2.1 OPERATIONAL PLANS

**Operational plans** that are short-term and deal with the day-to-day work of their team. Short-term goals are aligned with the long-term goals and can be achieved within one year.

Operational plans include policies, procedures, methods, and rules an operational plan is an annual work plan

An operational plan is the basis for and justification of an annual operating budget request. Therefore, a strategic plan that has a five-year lifetime would drive five operational plans funded by five operating budgets.

### 3.2.2. STRATEGIC PLANNING

Strategic Planning is process to establish priorities on what you will accomplish in the future. Forces you to make choices on what you will do and what you will not do. Pulls the entire organization together around a single game plan for implementation. Broad outline on where resources will get allocated.

A strategic plan is a road map to lead an organization from where it is now to where it would like to be in five or ten years.

**Strategic planning typically examines an organization's purpose, mission, philosophy, and goals in the context of its external environment.**

Strategic planning then forecasts the future success of an organization by matching and aligning an organization's capabilities with its external opportunities "helps to separate daily management activities and operations from breakthrough initiatives that can take the organization or unit to the next level."

### 3.3. SWOT ANALYSIS

There are many effective tools that assist in strategic planning. One of the most commonly used in healthcare organizations is SWOT analysis

Environmental assessment (situation analysis) by using a "SWOT" form.

The aim of any SWOT analysis is to identify the key internal and external

factors that are important to achieving the objective.

SWOT analysis groups key pieces of information into two main categories:

**Internal factors** – The *strengths* and *weaknesses* internal to the organization.

**External factors** – The *opportunities* and *threats* presented by the external environment

### 3.3.1. SWOT DEFINITIONS

**Strengths** are those internal attributes that help an organization to achieve its objectives.

**Weaknesses** are those internal attributes that challenge an organization in achieving its objectives.

**Opportunities** are external conditions that promote achievement of organizational objectives.

**Threats** are external conditions that challenge or threaten the achievement of organizational objectives.




**Figure 3.1 SWOT analysis**

### 3.3.2 Internal assessment

**Looks for (strengths and weaknesses)**

**Strengths:** Management development, qualifications of staff, medical staff expertise, facilities, location, quality of services.

**Weaknesses:** Scarcity of staff, financial situation, cash flow position, and marketing efforts.

### 3.3.3 External assessment

**Looks for (opportunities and threats)**

**Opportunities:** nurse recruitment, physician recruitment, referral pattern, new programs, new markets, diversification, population growth, improved technology, and new facilities.

**Threats:** Shortage of nurses, decrease in patient satisfaction, decrease in demand services, competition, regulations, unionization, and loss of accreditation.

### 3.3.4. SWOT ANALYSIS, THEY SHOULD INCLUDE

(1) working out what you hope to achieve,

(2) being objective,

(3) incorporating the findings into an action plan, and

(4) revisiting your findings on a regular basis."

In essence, they suggest that honesty, specificity, simplicity, and self-awareness are integral to successful SWOT analysis.

There are several simple rules must be followed for SWOT analysis to be successful:

* Be realistic about the strengths and weaknesses of your organization.
* Be clear about how the present organization differs from what might be possible in the future.
* Be specific about what you want to accomplish.
* Always apply SWOT in relation to your competitors.
* Keep SWOT short and simple.
* Remember that SWOT is subjective

## 3.4. BALANCED SCORECARD

Balanced Scorecard is another tool that is highly assistive in strategic planning

Strategic planners using a Balanced Scorecard, develop metrics (performance measurement indicators), collect data, and analyze that data from four organizational perspectives:

1) Financial,
2) Customers,
3) Internal Business Processes (or simply processes), and
4) Learning and Growth.

Balanced Scorecards also allow organizations to align their strategic activities with the strategic plan.

The best-Balanced Scorecards are not a static set of measurements, but instead reflect the dynamic nature of the organizational environment.

Because the Balanced Scorecard is able to translate strategy into action, it is an effective tool for translating an organization's strategic vision into clear and realistic objectives.

## 3.5 STRATEGIC PLANNING AS A MANAGEMENT PROCESS

Regardless of the tool(s) used, strategic planning as a management process generally includes the following steps:

1.Clearly define the purpose of the organization.

2. Establish realistic goals and objectives consistent with the mission of the organization

3. Identify the organization's external constituencies or stakeholders and then determine their assessment of the organization's purposes and operations.

4. Clearly communicate the goals and objectives to the organization's constituents.

5.Develop a sense of ownership of the plan.

6.Develop strategies to achieve the goals.

7. Ensure that the most effective use is made of the organization's resources.

8.Provide a base from which progress can be measured.

9. Provide a mechanism for informed change as needed.

10. Build a consensus about where the organization is going.

**** Who Should Be Involved in Strategic Planning? ****

The first-level manager is generally more involved in long-range planning at the unit level. However, because the organization's strategic plans affect unit planning, managers at all levels must be informed of organizational long-range plans so that all planning is coordinated.

All organizations should establish annual strategic planning conferences, involving all departments and levels of the hierarchy.

The action should promote increased effectiveness of nursing staff, better communication between all levels of personnel, a cooperative spirit relative to solving problems, and a pervasive feeling that the departments are unified, goal directed, and doing their part to help the organization accomplish its mission.

## 3.6 THE PLANNING HIERARCHY




### 3.6.1. VISION AND MISSION STATEMENTS

**Vision**: It is a mental image or the power of imagination to see something that is not actually visible. the preferred future

Vision statements are used to describe future goals or aims of an organization. It is a description in words that conjures up a picture for all group members of what they want to accomplish together

Vision statement should require all organization members to stretch their expectations, aspirations and performance

E.g.: Al Shifa Medical Hospital will be the leading center for trauma care in Palestine

**An organization will never be greater than the vision that guides it.**

The purpose or mission statement is a brief statement (typically no more than three or four sentences) identifying the reason that an organization exists.

Most nursing services are existing to provide high-quality nursing care to their clients. Some are encouraging education and research.

The mission statement identifies the organization's constituency and addresses its position regarding ethics, principles, and standards of practice.

The mission statement is of highest priority in the planning hierarchy because it influences the development of an organization's philosophy, goals, objectives, policies, procedures, and rules.

EX: The purpose of in-service education department is to orient staff to the job and provide educational programs to improve the quality of staff work.

EX: The purpose of burn unit is to provide quality-nursing care to patients suffering from burns.

EX: The purpose of safe childbirth unit is to empowering midwife to provide quality care to improve maternal and neonatal health care.

An organization must truly believe and act upon its mission statement; otherwise, the statement has no value.

### 3.6.2. THE ORGANIZATION'S PHILOSOPHY STATEMENT

**The philosophy** flows from the mission statement and delineates the set of values and beliefs that guide all actions of the organization. It is the basic foundation that directs all further planning toward that mission.

A statement of philosophy can usually be found in policy manuals at the institution or is available on request. It should be written, placed in appropriate document and reviewed periodically.

There should be a philosophy for the institution and philosophy for each department (ICU, lab.)

### 3.6.2.1 BENEFITS OF PHILOSOPHY:

1. Translate writing into practice.
2. Lead to holistic approach in practice since hospitals are multi-disciplinary system.
3. Used as a justification of quality of service provided.
4. Provides framework for development of nursing care model (functional, primary, team).
5. Provides a form of control.
6. Deals with fundamental long-range goals and value.

A working philosophy is evident in a department's decisions, in its priorities, and in its accomplishments

### 3.6.3 VALUES

It is the worth, usefulness, or importance of something. The guiding value for general hospital is as follows Quality, Compassion, Fairness, Integrity and Innovation

All people should carefully examine their value system and recognize the role that it plays in how they make decisions and resolve conflicts and even how they perceive things. Most people have many value indicators but few true values.

McNally's identified the following four characteristics that determine a true value:

1. It must be freely chosen from among alternatives only after due reflection.

2. It must be prized and cherished.

3. It is consciously and consistently repeated (part of a pattern).

4. It is positively affirmed and enacted.

### 3.6.3.1 GUIDING PRINCIPLES AND VALUES

Every organization should be guided by a set of values and beliefs. Provides an underlying framework for making decisions – part of the organization's culture

Values are often rooted in ethical themes, such as honesty, trust, integrity, respect, fairness, . . .

Values should be applicable across the entire organization. Values may be appropriate for certain best management practices – best in terms of quality, exceptional customer service, etc.

### 3.6.4. GOALS AND OBJECTIVES

Goals and objectives are the ends toward which the organization is working They state actions for achieving the purpose and philosophy.

All philosophies must be translated into specific goals and objectives, if they are to result in actions

Goals defined as the desired result toward which effort is directed. It is the aim of philosophy.

**It should be:**

* Specific enough to clearly delineate the desired end product.
* Measurable and realistic.
* Change over time, so they require periodic re-evaluation and prioritization.

**Goal Should be: SMART**

**S: Specific**
**M: Measurable**
**A: Achievable or Attainable**
**R: Realistic**
**T: Time pounded**

### 3.6.4 OBJECTIVES

More specific and measurable than goals because they identify how and when the goal is to be accomplished.

They invite the individuals to a specific end and are explicit, measurable, observable and obtainable.

One goal is usually having many objectives (each is accompanied by a target completion date).

**Objectives should include:**

- ✓ Who will do the work?
- ✓ Write the word (To) followed by action verb.
- ✓ Write single key result to be achieved.
- ✓ Write the word (By) and the date for accomplishment.
- ✓ Write the maximum cost in terms of money or time.

### 3.6.5 POLICIES AND PROCEDURES

#### 3.6.5.1 Policy

Defined as broad general statement of expected actions that serve as guides to managerial decision making or to supervising the actions of subordinates.

They are plans reduced to statement or instruction that directs organization in their decision making. Policies & procedures: are means for accomplishing goals and objectives.

Polices use purpose, philosophy, goals and objectives as a guide. It directs individual behavior toward organization's mission and defines broad limits and desired outcomes of commonly recurring situations.

It serves as a basis for future decision and actions, help co-ordinate plans, control performance and increase consistency of actions by increasing the

possibility that different manager will make similar decisions when independently facing similar situation.

Policy formation is a continuous process so that should be re-evaluated and re-stated as necessary.

EX: annual leave, working hours, shifts, vacations, maternity leave.

It should be written, clear, every employee should know it. Policy gives feeling of fairness.

### 3.6.5.2 PROCEDURES

Are plans that have been reduced to sequence of steps of required action. Procedures identify process needed to implement a policy and generally found in manuals at unit level.

Procedure manual provides basis for orientation and staff development and ready reference. Procedure manual standardized procedure and equipment and can provide basis for evaluation. Good procedures reflect quality of care. Save times, facilitate delegation, reduce cost, and increase productivity, provide a mean of control.

### 3.6.6 RULES and REGULATIONS

Rules and regulations are plans that define specific action or non action. As part of policy and procedure statements, *rules* describe situations that allow only one choice of action. Rules are fairly inflexible, so the fewer rules, the better. Existing rules should be enforced to keep morale from breaking down and to allow organizational structure.

## 3.7 INTEGRATING LEADERSHIP ROLES AND MANAGEMENT FUNCTIONS IN PLANNING

The leader–manager must be skilled in determining, implementing, documenting, and evaluating all types of planning in the hierarchy because an organization's leaders are integral to realizing the mission of the organization.

Planning requires managerial expertise in healthcare economics, human resource management, political and legislative issues affecting healthcare, and planning theory.

Planning requires the leadership skills of being sensitive to the environment, being able to appraise accurately the social and political climate, and being willing to take risks.

*The final step in the process involves articulating identified goals and objectives clearly; this learned management skill is critical to the success of the planning. If the unit manager lacks management or leadership skills, the planning hierarchy fails.*

# Chapter 4

# TIME MANAGEMENT

*. . . nothing is particularly hard if you divide it into small jobs.*
–HENRY FORD

*. . . things which matter most must never be at the mercy of things that matter least.*

–JOHANN WOLFGANG VON GOETH

## LEARNING OBJECTIVES

• Analyze how time is managed both personally and at the unit level of the organization

• Describe the importance of allowing adequate time for daily planning and priority setting

• Complete tasks according to the priority level they have been assigned whenever possible

• Build evaluation steps into planning so that reprioritization can occur

• Identify common internal and external time wasters as well as interventions that can be taken to reduce their impact

• Complete a time inventory to increase self-awareness regarding personal priority setting and time management

Time is an equally important resource. Being overwhelmed by work and time constraints leads to increased errors, the omission of important tasks, and general feelings of stress and ineffectiveness. If managers are to direct employees effectively and maximize other resources, they must first be able to find the time to do so. In other words, they must become experts at time management.

Time management can be defined as making optimal use of available time. Good time-management skills allow an individual to spend time on things that matter.

Because time is a finite and valuable resource, learning to use it wisely requires both leadership skills and management functions. Leadership roles and management functions needed for effective time management.

## 4.1 THREE BASIC STEPS TO TIME MANAGEMENT

There are three basic steps to time management:

-The first step requires that time be set aside for planning and establishing priorities.

-The second step entails completing the highest-priority task whenever possible and finishing one task before beginning another.

-The third step, the person must reprioritize what tasks will be accomplished based on new information received.

Because this is a cyclic process, all three steps must be accomplished sequentially.

## 4.2TIME MANAGEMENT TIPS

1. Plan Your Day
2. Stop Procrastinating.
3. Prepare for Meeting.
4. Handle Paperwork.
5. Delegate

6. Say No.
7. Simplify your Life.
8. Reduce Stress.
9. Prevent Interruptions.

## 4.2.1. PLAN YOUR DAY

- Write out a To Do list every day.
- Separate your To Do list into A, B and C priorities.
- Start with the A items.
- Try to do your planning at the same time every day.
- Check off items as you complete them to give yourself a sense of accomplishment.
- Don't jam your day full of activities. Leave time for emergencies, thinking.
- Be your own manager.
- Always plan time for balance.
- Do it now.

**♦External Time Wasters.**

Be aware of ways others or the environment waste your time:

1-Interruptions.
2- Office socializing.
3-Too many meetings.
4-Unscheduled visitors.
5-Poor work environment.
6-Unclear goals.
7-Trying to get other's cooperation.
8-Bureaucratic "red tape".

**♦Internal time wasters**

**Be aware of ways in which you waste your own time:**

1-Procrastination.
2-Lack of planning.
3-Lack of priorities.
4-Slow reading skills.
5-Physical or mental exhaustion.
6-Not being able to say "no".
7-Low motivation.

## 4.2.2. STOP PROCRASTINATING

- It is a habit of postponement important things to do.
- Understand cause and develop strategy to fix it.
- Complete unpleasant tasks.
- Break large jobs in manageable tasks.
- Develop a clear mental picture of completed task and how you will feel.
- Remove distracters.

**♦Task Strategies:**

- **Unpleasant tasks:**

Complete these tasks first. Schedule them for early in the day. Give yourself a reward for doing them.

- **Complex Projects:**

Break large jobs into smaller, more manageable tasks.

- **Fear of the failure (lake of self-confidence):**

Develop a clear mental picture of the task and how you will feel at that time.

- **Lack of interest:**

Schedule the task for when you will be at you peak.

- **Perfectionism:**

Set deadlines for yourself. Tell other people your deadlines.

- **Distraction, lake of focus:**

Complete something. Make a very small task for yourself and finish it. very small then, make another one.

### 4.2.3. PREPARE FOR MEETINGS

- Make invitation at least two weeks in advance.
- Send a follow-up confirmation note.
- Preparing an agenda in advance.
- Start and finish with positive items.
- Consider starting meetings at oddball times.
- Schedule meetings for late in the day.
- Tendency to move quicker at the end of the day.

### 4.2.4. HOW TO HANDLE PAPERWORK

- Sort papers in your in box **ONLY** twice per day.
- Throw away previous draft.
- Sort out items according to urgency.
- Ask subordinates to submit recommendations along with reports.

- Create different file folders.
- Cut back on sending memos.
- Don’t keep copies of requests to others unless legal.

## 4.2.5. DELEGATION

- Assume all people want added responsibilities.
- Do not delegate what you can eliminate.
- Delegate things that aren't part of your core competency.
- Some things cannot be delegated, performance reviews, firing …
- Delegate do not abdicate.
- Delegate the objective, not the procedure.
- Ask people to provide progress report.
- Delegate to the right person.
- Spread delegation around and give people new experience as part of their training.
- Be sure to delegate the authority along with the responsibility.
- Trust people to do well

## 4.2.6. HOW TO SAY NO

- Stick to your plan.
- When someone persists repeat position in a different way.
- Saying no saves, you (time, cost).
- Be polite but firm.
- Provide suggestions or alternatives to the person who is asking.
- Remind them of what work load you have.

## 4.2.7. REDUCE STRESS

SPARKLE Formula:
**S**leep
**P**lan
**A**nticipate Less
**R**elax
**K**eep Anger Under Control
**L**augh
**E**at

## 4.2.8. PREVENT INTERRUPTIONS

- Prevent interruption from walk-in visitors by isolating yourself.
- Don't feel obliged to have “an open-door policy”.
- Block off your time for priorities.
- Inform co-workers or subordinates you generally like to come in at perhaps 8:00am, and work on your own until 9: 30am.only then do you accept meetings.

## 4.2.9. SIMPLIFY YOUR LIFE

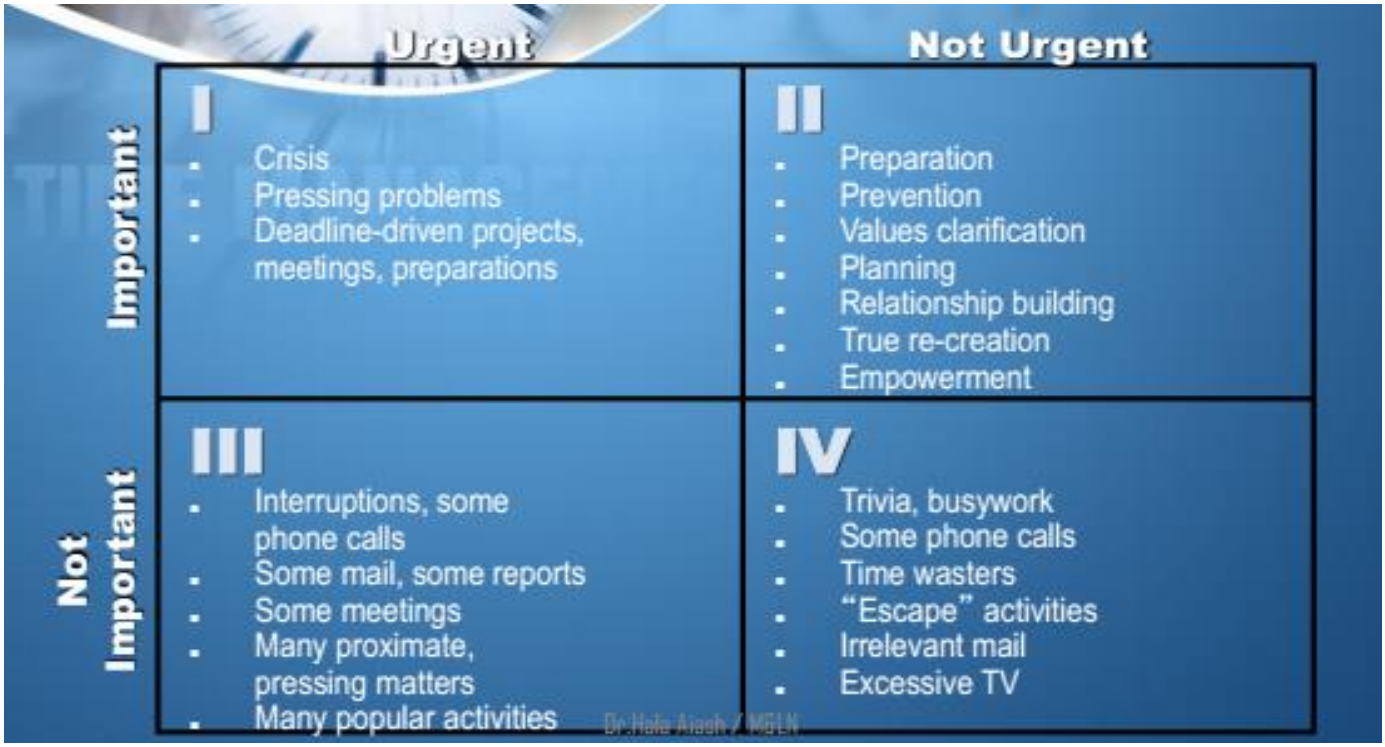
- Write a list of goals you'd like to achieve.
- Stop being a slave to communication tools.
- Cut back on television time.
- Make a plan for the weekend that doesn't involve work.
- Make time for yourself.
- Clean out your office.

## 4.3. PERSONAL TIME MANAGEMENT

Personal time management refers in part to self-knowledge. Self-awareness is a leadership skill. For people who are not certain of their own short- to long-term goals, time management in general, poses difficulties. Managing time is difficult if a person is unsure of his or her priorities for time management, including personal short-term, intermediate, and long-term goals.

## 4.4. THE PRIORITY MATRIX

| | |
|---|---|
| **(1)**<br>**Important**<br>**Urgent** | **(2)**<br>**Important**<br>**Not urgent** |
| **(3)**<br>**Not important**<br>**Urgent** | **(4)**<br>**Not important**<br>**Not urgent** |



## 4.5. THE RESULT OF GOOD TIME MANAGEMENT

- You complete your work in the allotted time.
- You meet deadline.
- The quality of your work improves.
- You can identify and manage your workload.
- You know what you are doing and why at any given time.
- Everyone else knows what you are doing and you know what everyone else doing.
- You have a sense of satisfaction
- Stress level is decreased.

# Chapter 5

# Roles and Functions in Organizing

***. . . The days of the traditional pyramid shaped corporate hierarchy as a viable business model are coming to an end***

**—MICHAEL HUGOS**

***. . . in a changing world, organizations must change as surely as individuals must change. Recent years have seen an increase in organizational "flattening," the tendency to shrink the organizational structure through the removal of layers of hierarchy.***

**—CHARLES R. MCCONNELL**

## LEARNING OBJECTIVES

-Describe how the structure of an organization facilitates or impedes communication, flexibility, and job satisfaction

-Identify line and staff relationships, span of control, unity of command, and scalar chains on the organization chart

-Differentiate between first, middle, and top levels of management
-Compare and contrast centralized and decentralized decision making
-Analyze how position on the organization chart is related to centrality
-Describe at least one model of shared governance and differentiate it from participatory decision making.

**Organizing** is the second managerial function, having planned, the manager must now organize so that personnel can accomplish the plan with efficiency and effectiveness.

**Organizing** activities include grouping people and activities and placing essential people where they will be most useful.

**Definition:** an organization is a collection of people working together under division of labor and a hierarchy of authority to achieve a common goal.

The organizational structure should provide an effective work system, a system of communication identity to individuals and to the organization…. and should consequently foster job satisfaction.

Systems have input, processes, outputs and outcomes, with ongoing feedback among these various parts.

## 5.1 ORGANIZATIONAL STRUCTURE

Agencies contain both formal and informal structures. Formal structure appears on the organization chart. It can be diagramed to show the relationships among people and their positions. A formal structure describes position, task responsibility and relationship.

Informal structure comprises personal and social relationship that don't appear on the organization chart (it is based on personal relationship rather than positional authority).

Informal authority is not commanded through organizational assignment. It comes from the followers natural respect for a colleagues knowledge and abilities.

## 5.2 ORGANIZATIONAL CONCEPTS

Organizational Charts: is a drawing that show how the parts of an organization are linked, it depicts the formal organization at relationship, area of responsibility, persons whom one is accountable, and channels of communication. ((Fig. 12.1)

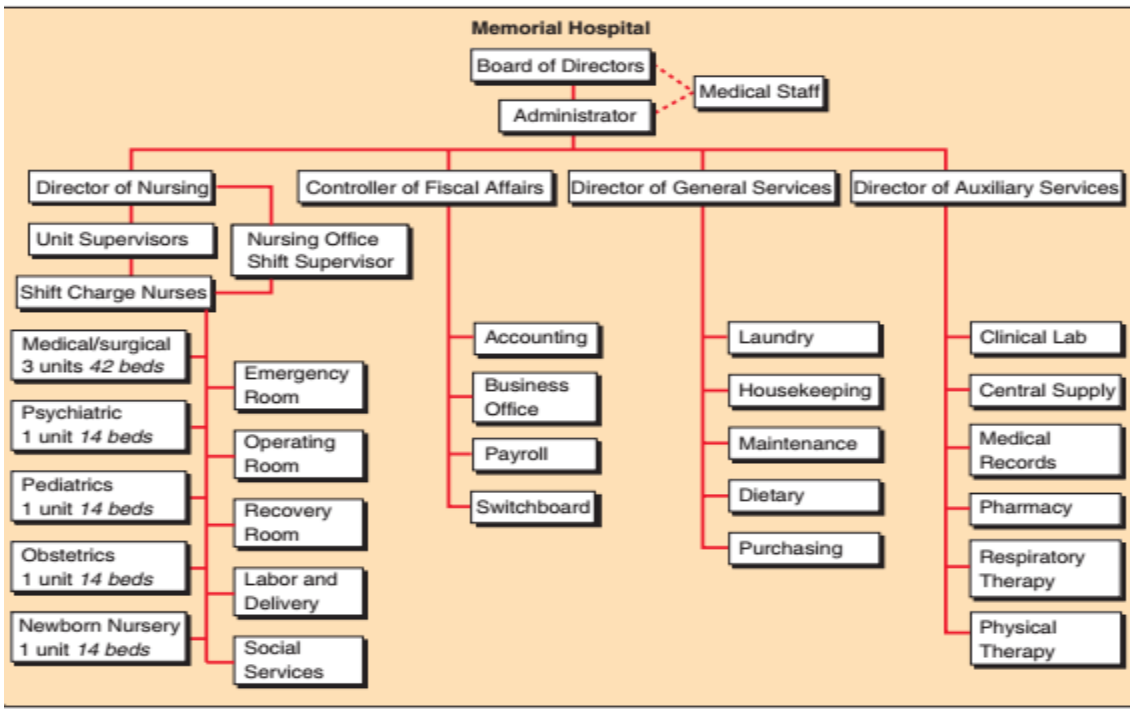



**Figure 12.1** Sample organizational chart.

**Span of control (management):** is the number of people reporting to any manager represents the managers' span of control and determines the number of interactions expected of him or her.

**Accountability**- it is an agreement to accept the consequences of your actions.

**Responsibility**- The duty to carry out an assignment or conduct a certain activity.

**Authority**: is the official power to act. Given by the organization to direct the work of others.

## 5.3 Management Levels?

| | Top Level | Middle Level | First Level |
|---|---|---|---|
| Examples | Chief nursing officer<br>Chief executive officer<br>Chief financial officer | Unit supervisor<br>Department head<br>Director | Charge nurse<br>Team leader<br>Primary nurse |
| Scope of responsibility | Look at organization as a whole as well as external influences | Focus is on integrating unit level day-to-day needs with organizational needs | Focus primarily on day-to-day needs at unit level |
| Primary planning focus | Strategic planning | Combination of long- and short-range planning | Short-range, operational planning |
| Communication flow | More often top down but receives subordinate feedback both directly and via middle-level managers | Upward and downward with great centrality | More often upward; generally relies on middle-level managers to transmit communication to top-level managers |

## 5.4 TYPES OF NORMAL ORGANIZATIONAL STRUCTURES:

Most nursing departments utilize one of the following structural patterns:

1. Line organization
2. Functional structure
3. Dual Structure
4. Self-contained unit structure

### 5.4.1. Line organization:

- ♠ Most common structure in large health care facilities.
- ♠ Line positions can be shown either horizontal or vertical unbroken line

**A. Horizontal unbroken line:**

- ♠ Represents communication between individual with similar spheres of responsibility and power but different functions.

**B. Vertical unbroken line:**

- ♠ represents communication between individuals with different positions.
- ♠ Those having the greatest decision-making and authority are located at the top, and those with the least at the bottom.

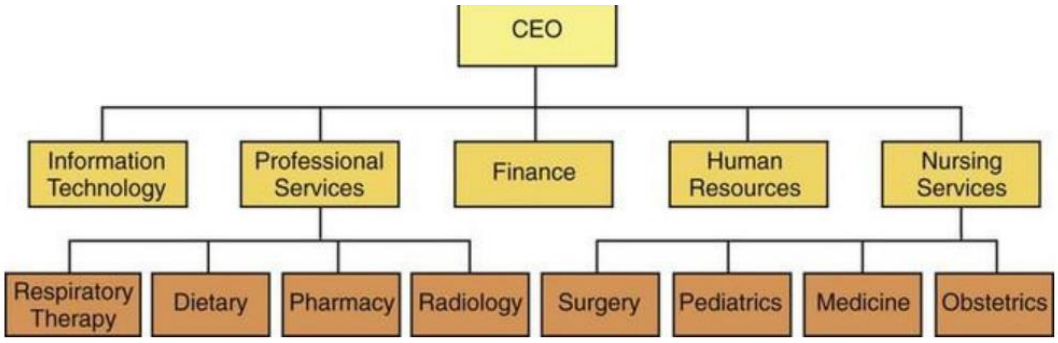
**C. Dotted or broken lines:**

- ♠ Represent staff position.
- ♠ These positions are advisory…... a staff member provides information and assistance to the manager but has limited authority.
- ♠ These positions provide for specialization that would be impossible for anyone manager to achieve alone.

### 5.4.2. Functional structure

- ♠ Employees are group in departments by task.
- ♠ Similar task in the same group … similar group in the same departments … and similar departments report to the same manager.

- ♠ All nursing task are under nursing service and the same is true for other functional areas.




**Advantages:**

- ♠ **Use resources efficiently.**
- ♠ **Do not duplicate tasks.**
- ♠ **Simplify training because common tasks are grouped together.**

**Disadvantages:**

- ♠ **Coordination across function is poor and difficult.**
- ♠ **Response time is slower**

### 5.4.3. Dual Structure:

- ♠ Separates technical and administrative responsibilities.
- ♠ It has one hierarchy for technical professionals who make technical decisions and control technical matters and another administrative hierarchy for supervisory management who make decisions about issues such as personnel and budget.

**Advantages:**

- ♠ Give equal status to technical and administrative managers

### 5.4.4. Self contained unit structure:

- ♠ All functions needs to produce a product or service are grouped together in an autonomous division.

- ♠ Large health care organizations that acquire a smaller clinic may operate it as a self-contained unit.

**Advantages**:

- ♠ High client satisfaction can be achieved (save time).
- ♠ High coordination across function occurs (employee collaborate with other unit functions to meet unit goals and reduce conflict).

**Disadvantages**:

- ♠ Each service line is independent has its own nursing staff and compete with other service lines.

## 5.5. CENTRALIZATION VERSUS DECENTRALIZATION

- ♠ In a highly centralized organization, the chief executive makes most decisions. Decentralization is the allocation of responsibility and authority for management decisions downward through the chain of command. In centralized, decisions made at the apex of the organization takes longer period of time than decisions made at the lower levels. Therefore, highly centralized organizations are slow in adapting to major changes. Lower level workers become passive, unenthusiastic and mechanical.
- ♠ The executive who will not permit supervisors to select staff, determine staff schedule, institute working improvements, evaluate goal achievement, and recommend policy change deprive middle managers of opportunities for professional growth.
- ♠ Decentralization of responsibility leads to improved employee morale. When middle managers are given responsibility for decision-making, they in-turn make still further, empowering staffs to formulate unit level work plans, policies and procedures.
- ♠ As job responsibility and autonomy increase, so does job satisfaction. It improves staff nurse moral and retention. The head nurse's tasks in a decentralized organization are similar to a nursing director task in a highly centralized organization. Therefore, expert staff specialist should support the head nurses.

## 5.6 HOW TO KEEP ORGANIZED?

Every plan should be flexible enough to allow for changes if the need arise, So if rearrangement of duties become necessary, you need to evaluate the situation, change as a little possible in order to minimize confusion and chance for error.

**Help your team to keep organized by:**

- ♠ Giving each person time and opportunity to plan the work.
- ♠ Have all necessary supplies available.
- ♠ Avoid interrupting your team except emergency.
- ♠ Encourage and help each member to improve his work habit.
- ♠ Help team to evaluate their work.
- ♠ Evaluate the effectiveness of you plan.
- ♠ Decide whether the work had been completed on time.
- ♠ Determine if your patients are satisfied with the care are received.
- ♠ Determine your team member's satisfaction.

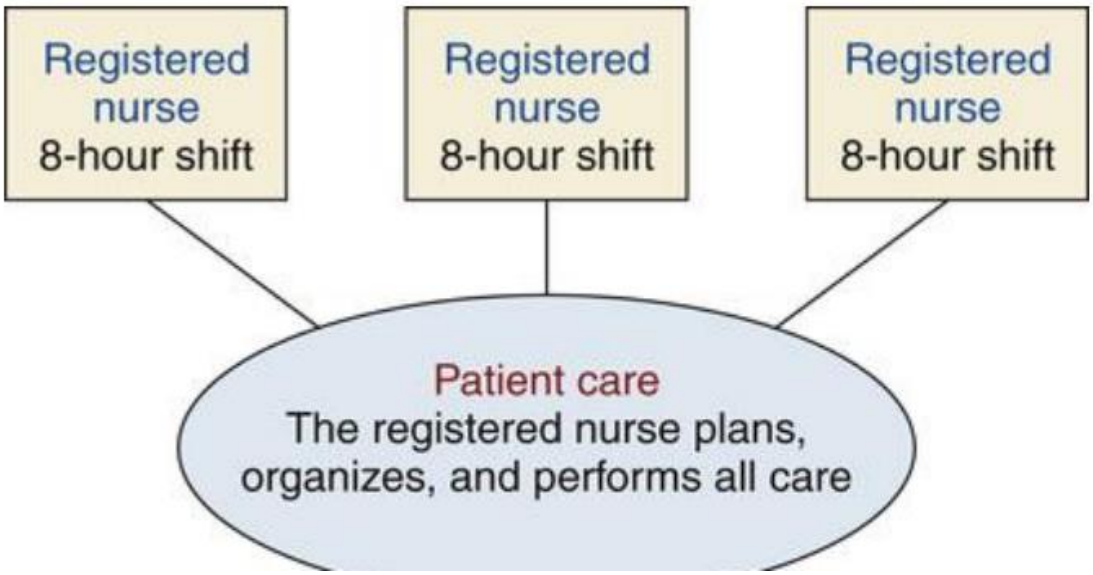
## 5.7. ORGANIZING AND DELIVERY OF NURSING CARE

- ♠ Nursing care delivery system refers to the manner in which nursing care is organized and provided.
- ♠ Effective delivery of nursing care promotes efficiency in an organization through high productivity and adequate staffing.
- ♠ Effective delivery of nursing care helps in increasing job satisfaction.
- ♠ The type of patient's care delivery system used reflecting the organization structure, philosophy, it also depends on nursing staffing.

## 5.8. TYPES OF NURSING CARE DELIVERY SYSTEMS

A. **Case Method:**

- ♠ Involves one-to-one nurse patient ratio.
- ♠ One nurse is responsible for caring for one patient and providing all the care required.
- ♠ The responsible nurse reports to the head nurse.
- ♠ Although this approach is expensive, it continues to be used in critical care units.




**Advantages:**

- ♠ Based on holistic philosophy of nursing.
- ♠ The nurse is fully responsible about the patient and gives him full accountability.
- ♠ Direct contact with the patient.
- ♠ Continuity of patient's care.

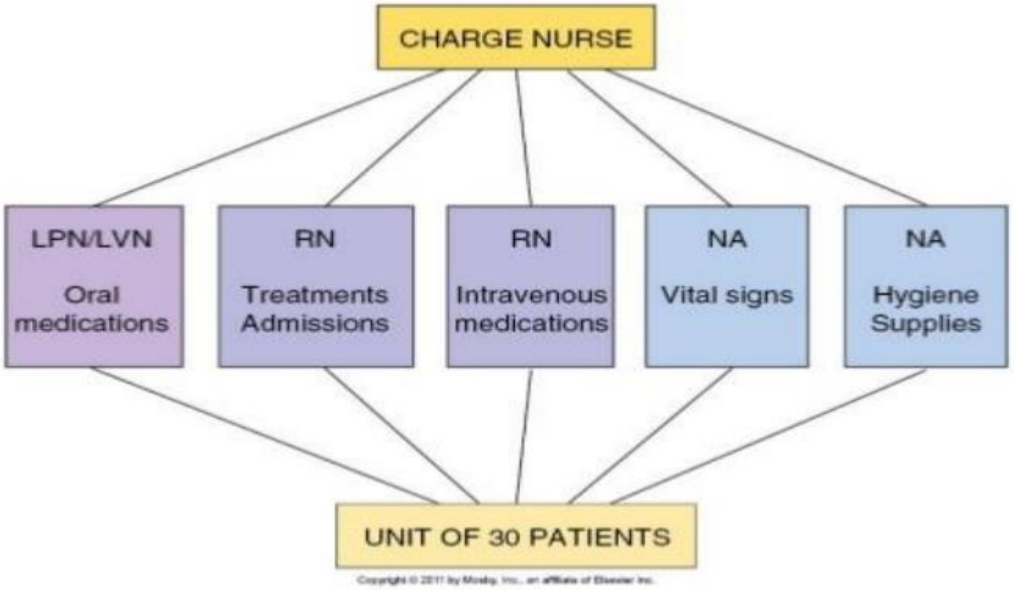
- ♠ Good nurse-patient relationship.
- ♠ Allow the nurse to coordinate patient's care (authority).

**Disadvantages:**

- ♠ Lack of educational development.
- ♠ Very expensive.

**B. Functional Method:**

- ♠ Fragmented approach, focuses on task and procedure.
- ♠ Tasks are assigned to various personal based on complexity and required skills.
- ♠ Each staff is responsible only for assigned task …. One nurse for medication …. Another one for dressing …. Another one for vital signs ….
- ♠ The charge nurse is responsible for coordinating the activities and report to the nurse manager.



**Advantages:**

- ♠ Based on task-oriented philosophy of nursing.
- ♠ Not expensive in comparison with case method.
- ♠ Efficient because tasks are completed in reasonable amount of time.
- ♠ Staffs do only what they are able to do and trained to do.
- ♠ Helpful in critical shortage of nurses / emergency.

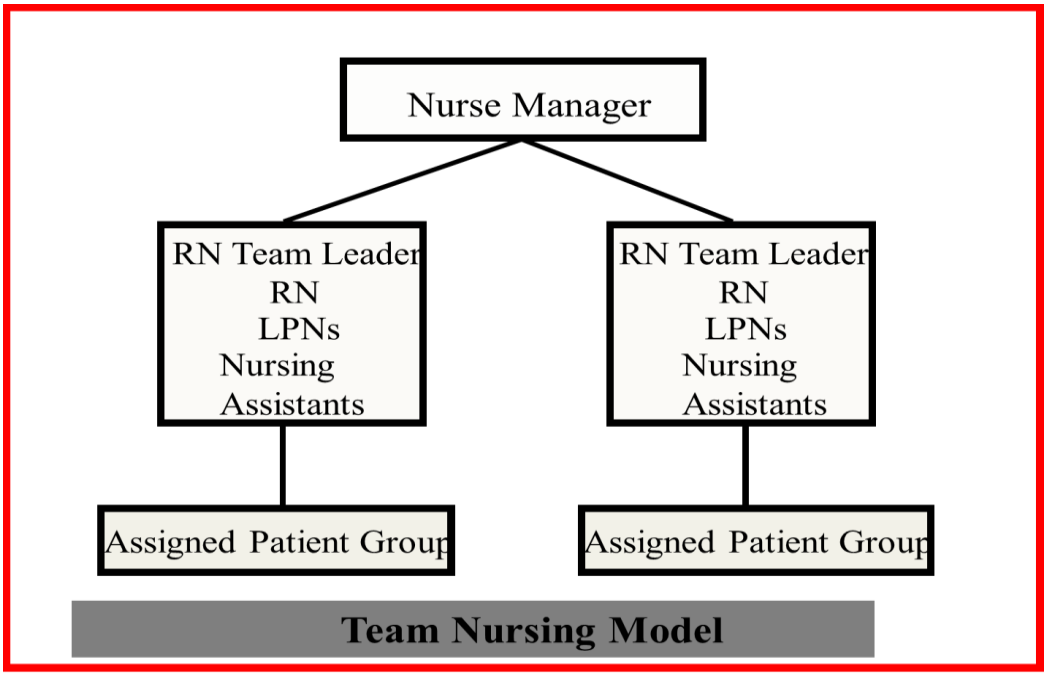
**Disadvantages:**

- ♠ Fragmented nursing care.
- ♠ Patients do not know their nurses.
- ♠ Nurses do not have enough time to communicate with patients.
- ♠ There is a little time for psychological care.
- ♠ Deals with patients in a mechanistic way rather than humanistic.
- ♠ Poor job satisfaction.
- ♠ Reflects bureaucracy and centralized organ.

**C. Team Method**

- ♠ Based on group philosophy.
- ♠ Reflect decentralization…attempt to support goal action through group action.
- ♠ The team leader is responsible for managing the care of group.
- ♠ Working with the team leader various qualified personnel, they report to the team leader, who then report to the head nurse.
- ♠ The team leader assigns personnel based on qualification and client needs.

- The team leader is responsible for planning and evaluating the nursing care provided by the team members.




**D. Team Method**

The nurse manager remains responsible for major management, decisions, communication and coordination.

**Advantages:**

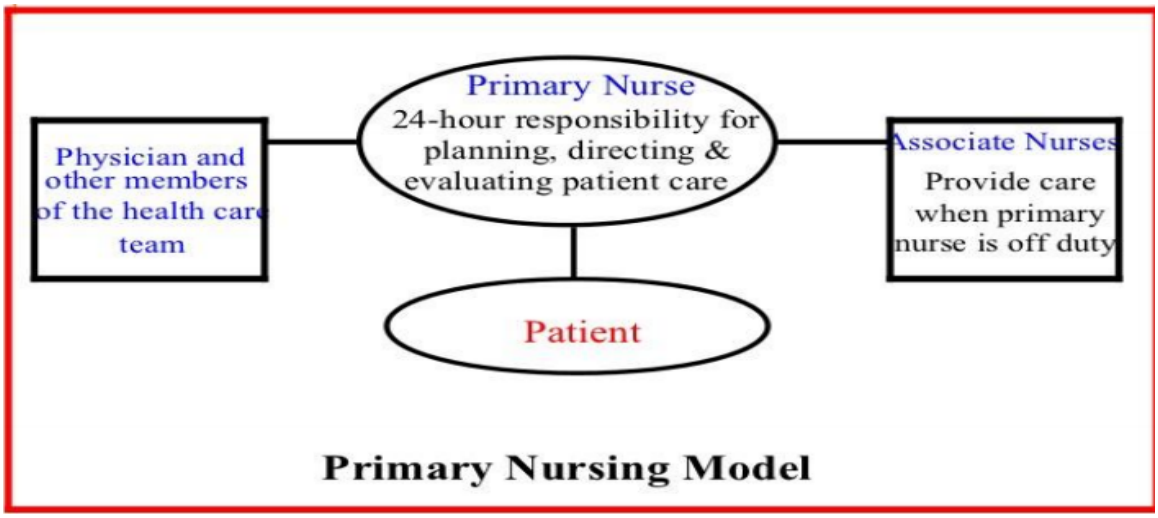
- High job satisfaction.
- Nurses have enough time to communicate.
- Patients know their nurses.

**Disadvantages:**

- Required efficient team leader skills.
- Rotation of team leader is frustrating.

## E. Primary Method

- ♠ Reflect decentralized structure and based on comprehensive philosophy.
- ♠ A primary nurse is assigned to care for group of patients 24hrs a day through their stay in the hospital.
- ♠ He has 24hrs responsibility for assigned patients.
- ♠ Responsible for assessing, planning, implementing and evaluating the nursing care.
- ♠ The nurse manager is responsible for assigning primary nurses, coordinating the activities of primary nurse on all shifts and assigning associate nurses for periods when primary nurses are of duty.




**Primary Nursing Model**

## E. Primary Method

Associate nurse carry out the plan of care when the primary nurse is not available.

**Advantages :**

- ♠ Provide satisfaction for nurses and patients.

- ♠ Nurses get self esteem for providing complete nursing care.
- ♠ Good communication between patients and nurses.
- ♠ Increase accountability and facilitate continuity of care.
- ♠ Efficient method which decrease the number of people in the chain of commands (reduce error).
- ♠ Patient know their nurses and times they are available.

**Disadvantages**:

- ♠ Primary nurse limits a nurse's talents to her own patients, other patients can't get benefit.
- ♠ More professional staff and fewer non-professional staff are required for successful implementation.
- ♠ Complicates doctors rounds because no single nurse in the unit knows all the patients.

# Chapter 6

# Management of Change

## LEARNING OBJECTIVES

1. Describe the importance of change and appreciate its nature
2. Explain key factors in management of change

3. Discuss the source of resistance to change and the ways this resistance can be overcome

4.Play a leading role in initiating and managing change in your organization of work environment

5. Discuss Lewin's 3-step model of change management.

6.Discuss the strategies for effective change.

7. List the techniques for dealing with resistance.

## 6.1 Introduction

Change is a major characteristic in all activities of modern societies. It has been said that “When we talk about future, we are discussing change”. “Change is the essence of life; it is as long as life goes on”.

Nowadays, Nurses are seeing many changes in the field of patient’s care and health care agency management. New medications, new equipment and new methods of treatment are being introduced to the health care centers at rapid rate. Nurses face many changes result changes on the political situation

## 6.2 DEFINITION

▪ Change: is the process of making something different from what it was.

▪ Change Agent: is one who works to bring about change

▪ Change is defined as "the process of alteration or transformation of individuals, groups, and organization undergo in response to internal factors."

▪ But we tend to resist the changes introduced by management that affect our interpersonal and job relations.

### 6.2.1 Why do we resist these changes?

▪ Because they tend to threaten the security of the patterns to which we have grown accustomed.

▪ In order for health care agencies to meet the increasing demands for patient care services, changes in management of personnel must occur.

▪ The ability to introduce such changes with a minimum resistance is a key managerial skill.

### 6.2.2 Why individuals resist change?

1.Change may make an employee feel that he will no longer be needed and therefore lose his job (threatened self- interest).

2.It is easier not to change (Employees are accustomed to their routines/procedures)

3.The uncertainty that change brings.

4.The reason for the change may not be understood (inaccurate perception).

5.Change may upset the interpersonal relationships established prior to the new system (objective disagreement)

6.Fear of failure, employees may feel that they are doing their jobs well now and that the change will alter their ability to work as well as (psychological disagreement)

## 6.3. PURPOSE OF CHANGE

▪ To meet changing clients' needs

▪ To meet changing market conditions

▪ To respond to internal pressures

▪ To take advantage of new opportunities

▪ To respond to competitive pressure

## 6.4 TYPES OF CHANGE:

### A. Planned change:

▪ Deliberate effort to make something happen.

▪ This type requires the leadership skills of problem solving and decision-making as well as interpersonal competence (the ability to work well with others as a group and on one to one basis).

### B. Accidental changes:

▪ Change by drift …. Unplanned. Occurs because of an imbalance in the system. Rules for implementing the change:

▪ Change should whenever possible be implemented for positive reason. This is not always possible in reality, as it is often enforced from above.

▪ As manager, it is important to try and avoid this type of change.

▪ Much change fails because it is not implemented for the right reasons, it occurs too suddenly, individuals affected are not involved or there is not enough planning.

## 6.5 LEWIN (1951):

identified the following rules that should be followed when implementing change:

1). Change should be only implemented for good reasons. Change for change's lead to unnecessary stress for employees.

**There are 3 good reasons for change:**

▪ Change in order to solve a problem.

▪ Change to make working procedures more efficient.

▪ Change to reduce unnecessary workload.

2). Resistance should be anticipated and natural part of the change.

Some theories perceive resistance as the most characteristic response to the change process.

**Resistance should be:**

▪ Covert: such as delaying tactics or passive behaviors.

▪ Overt: openly refusing to follow commands.

3.) Whenever possible, change should be planned and implemented gradually, not suddenly.

• Because change is such a complex process, it requires a great deal of planning and enough timing.

• Any change must allow enough time for those involved to be fully assimilated in that change.

4.) Those who may be affected, should be involved in planning for that change.

• Good, open communication throughout the process can reduce resistance.

• The easiest way for the manager to assure that subordinated accept the change is to involve them in the change process.

• When information and decision-making are shared, subordinates feel that they have played a valuable role in the change.

## 6.6. THE CHANGE PROCESS:

The change process can be divided into 3 main stages:

1.Planning.

2.Implementation.

3.Evaluation.

### 6.6.1. Planning

The planning process should involve:

▪ Identifying the reasons for change.

▪ Identifying key change agents.

▪ Communicate with and involve those who are affected by the change.

The suggested steps for planning the change are:

▪ State the goal: simple, concise terms should be used. Everyone should understand, accept and willing to work with the objectives.

▪ List alternatives: consider as many ways as possible to meet the objectives.

▪ Develop assumptions: each alternative is based on assumption. Based on these assumptions, the planner predicts the possible outcomes of each alternative.

▪ Choose an alternative: the best alternative that meets the objectives should be selected.

▪ Develop plans: once an alternative has been chosen, both strategic (long term) plans and operational (short term) planning is begun. Action plans for implementing, monitoring, and evaluating are developed.

▪ Put plans into action: effective action plans provide both short term and long-term directions for activity. This step includes the step of evaluating the alternative and planning for improvements.

**There are 3 main extrinsic factors to be considered when planning for change:**

1- People factor or human resources: include aspects of communication, attitudes, beliefs and values and leadership skills.

2- Structural factors: such as hierarchy, policies, procedures and politics.

3- Technological factors: includes the equipment and process that assist people to do their job.

▪ The employees must be informed about the reasons for the change in order to gain their support … any attempt to make them feel involved and consulted will have a positive effect in outcome of the change process.

▪ When planning change, it is important to identify the issues likely to cause conflict. Have key people on your side facilitating the change is helpful in persuading the rest of the employees.

### 6.6. 2. Implementation

▪ All staff should by now be aware of the reason for the proposed change and many of them involved in the planning.

▪ Set an agenda for stages of the change to be implemented if it is a big change.

▪ Give staff the opportunity to voice concerns, and let them know of a fixed evaluation date when they can discuss what has happened.

▪ Use key change agents where possible to assist and support in the implementation.

▪ Ensure that communication channels are open and everyone is aware of what is expected of them and what is trying to be achieved.

▪ Be flexible and ready to adjust or revise plans.

▪ Focus on positive aspects of change but do not ignore the negative.

### 6.6.3. Evaluation

▪ After an agreed period of time, the implemented change should be evaluated. This should involve everyone who is affected or had a role in planning the change.

▪ Outstanding problem can be raised, the effectiveness of the change can be examined and goals re – set if necessary.

▪ If change fails, reasons/causes need to be identified and the planning process may to ….

▪ If change fails, reasons/causes need to be identified and the planning process may need to re-start again. At this point staff who have supported the change need positive reinforcement.

▪ Staff who still resisting the change need to be tackled and reasons for their resistance tackled.

## 6.7 Changes Theories (Lewin's theory)

▪ Lewin provides a social- psychological view of the change process.

▪ He sees behavior as a dynamic balance of forces working in opposite directions within a field.

• Driving Forces: facilitate change.

• Restraining forces: forces that block or avoid the change.

▪ To plan change, one must analyze these forces (change agent begins the change process by analyzing the entire system in order to identify the forces against and forces with the change)

## 6.8. **Stages of Lewin's theory**

Lewin's identify three phases through which change takes place.

### 6.8.1. Stage of unfreezing:

▪ Unfreeze the existing equilibrium.

▪ Gather data: there is a problem and need change.

▪ Actively participating in problem identification and alternate solutions. And Motivate participants by getting them ready to change.

▪ People become dissatisfied and aware of a need to change.

▪ Build a trust and recognition for the need to change

Three mechanism for Unfreezing stage:

1- Lack of confirmation (disconfirmation): staff feel un comfort or dissatisfied with present (focus on negative point).

2- Guilt and Anxiety: its raise the tension level (reduce resistant).

3- Psychological safety: Its providing sufficient security to minimize the risk, many changes are resisted result threat. If the threat is removed or reduced, people will feel comfortable and attempt to change.

**6.8.2. Stage of Moving (Changing):**

▪ This is the implementation phase of change, the leader put the planned change into effect.

▪ Move the target system to a new level of equilibrium.

▪ The change agent identifies, plans and implements appropriate strategies, ensuring that driving forces exceeds restraining force

**The activities needed for a leader to engaged during the changing phase:**

1. Get participants to agree that the equilibrium status is not beneficial to them.

2. Develop a plan include everyone who will be affected in planning.

3. Set goals and objectives.

4. Identify areas of support and resistance.

5. Set target dates.

6. Introduce any new information that is needed to implement the change.

7. Encourage the new behavior so that it becomes part of the system's behavior.

8. Continue to provide a supportive climate to avoid resistance to change.

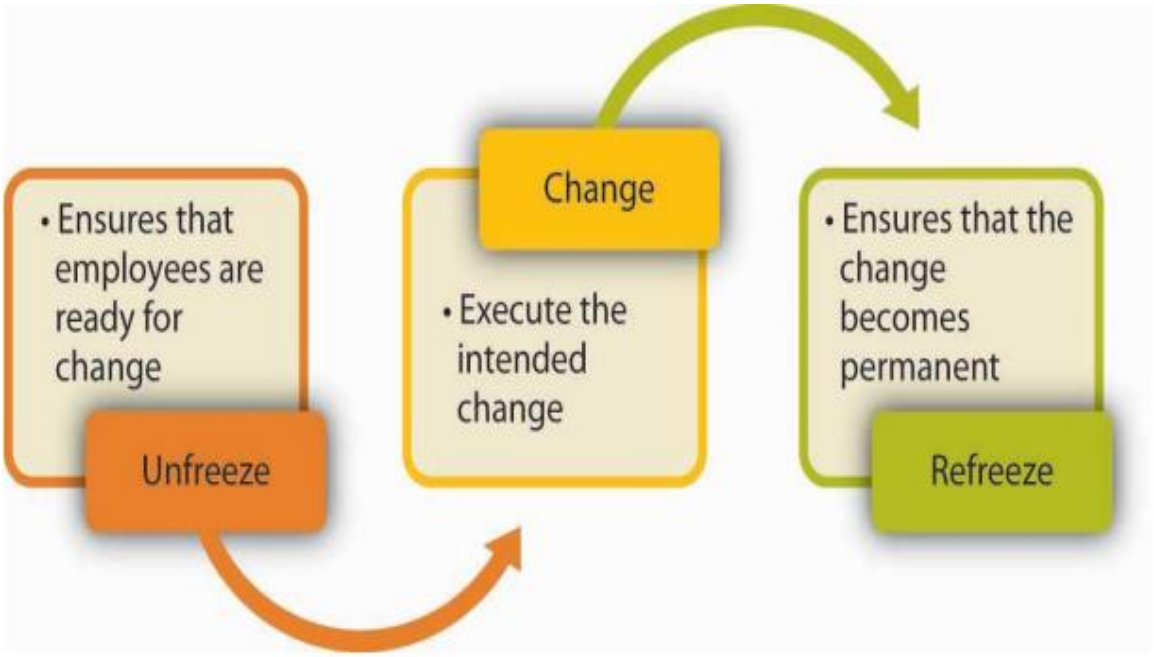
9. Provide feedback on progress and clarification of goals to reinforce the change process.

### 6.8.3. Re-freezing Stage:

▪ The change agent assists in stabilizing the change in the system.

▪ In order to facilitate the integration of the planned change the leader continue to act as energizer, guide the new behavior and increasingly delegate responsibility for change behavior to other people in the target system.

Creating an imbalance within the system by either increasing the driving forces or decreasing the restraining forces is a major task required for a change agent.



**The following examples illustrate how managers can change some of these targets.**

| Target | Example |
|---|---|
| **Individual** | • Fire a person and replace him/her with someone else.<br>• Change knowledge, skill, attitude, or behavior |
| **Technology** | • Replace existing technology with a more modern machine or way of doing work |
| **Structure** | • Change from functional structure to a product division structure<br>• Add a new department or division, or consolidate the existing ones |
| **Processes** | Change the pay system from hourly wages to salaries |
| **Culture** | • Implement a program to encourage valuing quality and service |
| **Management** | Encourage participation in the diagnosis and solution of problems by people at lower levels to replace a top-down approach |

## 6.9. STRATEGIES FOR EFFECTIVE CHANGE

The appropriate strategies for any situation depend on the power of the change agent and the amount of resistance expected from the subordinate.

**Strategies:**

1. Rational empirical strategies.

2. Normative re-educative strategies.

3. Power coercive strategy.

### 6.9.1. Rational empirical strategies

▪ This strategy based on the assumption that people are rational.

▪ This type of strategy is used when there is little anticipated resistance to the change or when the change is perceived as reasonable.

▪ But people not always act rationally

### 6.9.2. Normative re-educative strategies.

▪ This is the participative strategy.

▪ Based on the assumption that people act according to their commitment to sociocultural norms.

▪ These strategies use group norms to socialize and influence individuals, so change will occur

▪ The change agent assumes humans are social animals, more easily influenced by others than by facts. No need for legitimate power

### 6.9.3. Power coercive strategy.

▪ Are based on the application of power by legitimate authority, economic sanctions or political clout.

▪ Individuals will only change when rewarded for the change or are forced by some other power-coercive method.

▪ This strategy is used when high resistance is expected and when immediate change is necessary.

▪ The individual will change because there is a desire to avoid threat, peer pressure or loss of job.

## 6.10. TECHNIQUES FOR DEALING WITH RESISTANCE

• Participation.

• Manipulation.

• Education.

• External agent.

• Incentives.

• Supportive behavior (training, counseling).

• Gradual change.

# Chapter 7

# Roles and Functions in Staffing

*Employee selection is so crucial that nothing else—not leadership, not team building, not training, not pay incentives, not total quality management—can overcome poor hiring decisions . . .*

—GERALD GRAHAM

*. . . I have always surrounded myself with the best people to do their jobs, because I do not want to learn what they already know better than I do.*

—SHIRLEY SEARS CHATER

## LEARNING OBJECTIVES

1. Define staffing, human resource management and staff development
2. Discuss the main components of staffing (Human Resource Management)
3. Explain the relationship between staffing and scheduling
4. List the different types and options of scheduling
5. Determine the criteria for making assignment
6. Explain how to divide nursing activities.
7. Discuss the main components of staff development
8. Identify the factors influencing the organization of staff development activities
9. Demonstrate the goals, ways and methods of staff development

## 7.1. STAFFING

Staffing is the third phase of the management process.

In staffing the leader\manager **recruits**, **selects**, **orients**, and promotes personnel **development** to accomplish the goals of the organization.

Staffing is an especially important phase of the management process in healthcare organizations because they are labor intensive (i.e., they require many employees to accomplish their goals).

## 7.2. HUMAN RESOURCE MANAGEMENT (HRM):

▪ HRM is a modern approach of managing people at workplace which focuses on acquisition, development, utilization and maintenance of human resource.

▪ Like personnel management, human resource management is also concerned with planning, job analysis, recruitment and selection, hiring and socialization, training, performance appraisal, rewarding, and compensation management.

## 7.3. STAFFING INCLUDES:

### 7.3.1. Planning for staffing:

Adequate planning to determine staffing needs is often a neglected part of the staffing process.

Managers should know the source of their nursing pool, how many students currently enrolled in local nursing schools , staff resignation periods.

### 7.3. 2. Recruitment:

It is the process of actively seeking out or attracting applicants for existing positions.

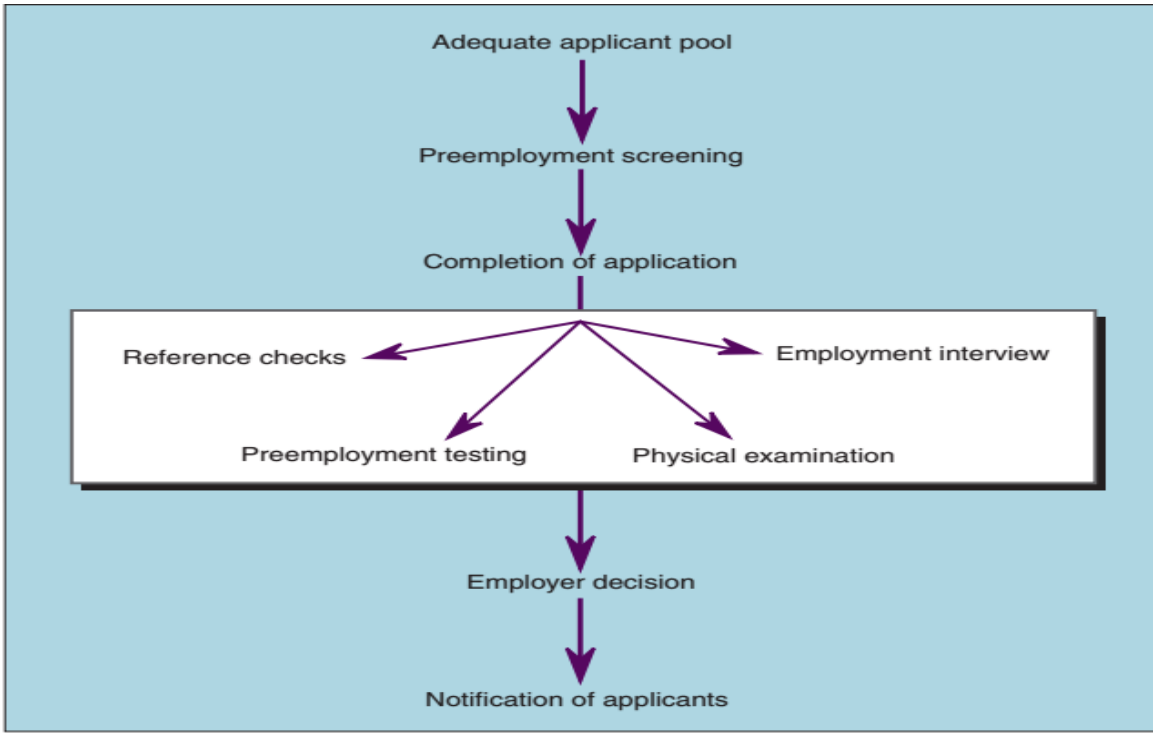
Before recruiting begins, organizations must identify reasons a prospective employee would choose to work for them over a competitor.

Recruiting includes: Advertising: in hospitals or journals for job vacancies.

## 7.3.3. Selecting:

▪ it is the process of choosing from among applicants the best-qualified individual or individuals for a particular job or position.

This process involves verifying the applicant's qualify cations, checking his or her work history, and deciding if a good match exists between the applicant's qualifications and the organization's expectations




**The selection process.**

## A.Educational and Credential Requirements

Screening job applicants (qualifications, experience) to ensure that the most appropriate candidates are hired.

## B.Reference Checks

All applications should be examined to see if they are complete and to ascertain that the applicant is qualified for the position. At this point, references are requested, and employment history is verified.

**C.Preemployment Testing**

Preemployment testing is generally used only when such testing is directly related to the ability to perform a specific job

**D.Physical Examination as a Selection Tool**

the medical exam is almost universal requirement for hiring .

these tests provide a means of determining if the applicant can meet the requirements for job.

**E.Making the Selection**

When determining the most appropriate person to hire, the manager must be sure that the same standards are used to evaluate all candidates.

Final selection should be based on established criteria, not on value judgments and personal preferences.

**F.Finalizing the Selection**

Applicants who are offered position should be requested to confirm in writing their intention to accept the position (sign a contract)

## 7.3.4. Placement:

▪ Appropriate placement is vital to organizations function and to the new employee's success, effectiveness, satisfaction and stability.

▪ Faulty placement can result in frustration of personal and professional ambition and can result in sacrifice to organizational efficiency and threats to organization integrity.

## 7.3.5. INDOCTRINATION

Indoctrination refers to the planned, guided adjustment of an employee to the organization and the work environment.

The indoctrination process includes three phases: induction, orientation, and socialization.

**Induction:**

The first phase of indoctrination, takes place after the employee has been selected but before performing the job role.

The induction process includes all activities that educate the new employee about the organization and employment and personnel policies and procedures.

**Socialization and orientation:**

▪ New employees should have an orientation to the organizations departments, rules and regulations, routines and introduction to other staff.

▪ Orientation period varies between different organizations and it takes one week usually.

## 7.4. STAFF DEVELOPMENT

The process of orientation, in-service education, and continuing education to promote development of personnel within any employment setting. It's part of the wider and longer-term strategy for the business and is aligned with the organization's objectives. Its purpose is to maximize the individual's potential by enabling them to improve their performance, to contribute to the continuous growth both of the organization and of themselves.

### 7.4.1. Staff Development Consist of:

In most organizations, staff development consists of three separates but interrelated activities:

**1. Orientation:**

Introducing new staff members to the philosophy, goals, policies, procedures, role expectation, physical facilities, and special services in a work setting.

**2. In-service education:**

Learning experiences provided in the work to assist staff in performing their assigned function.The ever-changing practice of health care requires continual updates. In service

education provides learning experiences intended to help staff members acquire, maintain, or increase competence in fulfilling expectations of employers regarding the application of technology equipment, medications, procedures and techniques in the specific work setting.

**3. Continuing Education:**

Educational programs that consist of concepts, principles, research, or theories related to nursing that build on previously acquired knowledge, skill and attitudes.

Continuing education Consist of professional learning experiences designed to enrich the employees' health care practice and contribute to quality health care. Continuing education builds on the learner current knowledge with information that is applicable to care provision in a wide variety of practice settings. All employees working in a health care setting have continuing educational needs. These needs are varying in degree and priority.

### 7.4.2. Training

Training is the planned process to modify attitude, knowledge or skills to achieve effective performance in a work-related activity. It is purpose in the work is to develop the individual's ability to satisfy the current needs of the organization.

### 7.4.3. Function of Development:

1- It is an accepted fact that development is necessary for the spirit, survival and performance of duties effectively now and in the future.

2- Development motivates employees to work harder. Nurses who understand their jobs are likely to have higher morale management gives them more confidence of their abilities to invest and provide assurance that they are valued nurses.

3- Development of staff level will elevate morale, confidence and value.

### 7.4.4. Component of Staff Development

Bill [1982] identifies three basic component of staff development:

**1- Inputs:** entry staff behavior, levels of education and prior experience.

**2- Through puts:** orientation, policies, procedures and managerial directions.

**3- Out puts:** the final staff behavior and quality of services.

### 7.4.5. Factors Influencing the Organization of Staff Development:

Hitching [1996] identified the following factors influencing the organization of staff development activities:

1- Philosophy and mission of the organization.

2- Organization of the nursing department.

3- Scope of staff development roles and responsibilities.

4- Educational preparation of nursing service executives, managers and staff nurses.

5- Educational preparation of the staff development director and educator.

6- Availability and use of clinical nurse specialists.

7- Diversification of patient care services.

8- Physical layout of the hospital.

9- Financial status of the institution.

### 7.4.6. Goal of Staff Development

The goal in nursing staff development curriculum development is to meet the continuing education needs of employees that will also move the organization toward its intended goals.

### 7.4.7. Ways Of Nursing Staff Development

1- Educational Days (study days).
2- Periodic seminars
3- Conferences Workshop.
4- Nursing rounds.
5- Field Trips (Home visit, assessment).
6- Exchange experiences.
7- Refreshing courses.
8- Case study.
9- Rotation of employee.

### 7.4.8. Staff Development Methods

**Internal Sources:**

Include on the job training, workshops and in-service programs.

**External Sources:**

Formal workshops by a training and education department and all training activities are done in college courses, conferences and continuing education workshops.

## 7.5. STAFFING AND SCHEDULING:

▪ Some health facilities decentralize staffing by having unit managers make schedules, other facilities use centralized stuffing with those schedules being made by single office or stuffing centers.

### 7.5.1. Decentralized staffing: the manager is responsible for:

▪ Covering all scheduled staff absences.

▪ Decrease staff during periods of decreased pt. census or acuity.

▪ Adding staff during increase number of pt.

▪ Preparing monthly unit report.

▪ Preparing holidays and vacations schedules.

▪ Order all needed supplies stores drugs and equipment….

▪ Team assignment.

**Advantages:**

▪ Nurses are more active in planning their own schedules and are able to make personal scheduling requests directly to their immediate supervisor.

▪ Decrease nurse attrition.

▪ Increase autonomy and flexibility.

▪ Increase retention of nurses.

**Disadvantages:**

▪ Unequal treatment, which result in negative staff reaction.

▪ Time consuming for manager.

### 7.5.2. Centralized staffing:

▪ Health care facilities with centralized staffing use one individual or computer to prepare the staffing and scheduling duties for all department.

**Advantages**:

▪ Fairness to employees.

▪ Relive head nurses from time-consuming duties and freeing them from other activities.

**Disadvantages**:

▪ Does not provide much flexibility for nurses.

▪ Increase absenteeism, conflict.

### 7.6. Scheduling options

▪ Rotating work shifts are common for staff nurses, the frequency of the alternating between days and evenings and nights varies among different organizations.

▪ Alternating and work shifts rotation creates stress for staff nurses and affect the health of nurses and the quality of their work.

#### 7.6.1. Type of schedules

**1- Permanent shifts:**

- ♠ Relives nurses from stress and health related problems associated with rotating shifts.
- ♠ Provide social, educational and psychological advantages.

**2- Block or cyclical scheduling:**

- ♠ use the same schedule repeatedly, every week or every month.

**3- Rotating schedules:**

- ♠ Moving between mornings, and evenings and nights during the week (common in hospitals).

**4- Variable staffing:**

- ♠ A method that use pts. needs to determine the number, mix of staff and timing.
- ♠ 8-hours shifts.
- ♠ 10-hours shifts.
- ♠ 12-hours shifts.
- ♠ Can use 2 hours between shifts
- ♠ Criteria for making assignment

## 7.7. Criteria for making assignment

### 7.7.1. Task related factors:

▪ The primary task-related concern is that the person assigned to the job has the ability to carry it out.

▪ Includes: ability, priority, efficiency and continuity.

**1. Ability:**

- Nurses should not be assigned tasks they can't do, regardless of their professional level.

- To make appropriate assignment, you need to know the **knowledge, skills and job description** for each one.

- Careful assessment of those characteristics is needed before making assignment.

**2. Priority:**

- Certain tasks are given priority over others.

**3. Efficiency:**

- Making procedures in an efficient way, taking in consideration values (**cost effect** consideration).

**4. Continuity:**

- continuity makes a more **holistic** approach and gives care-givers an opportunity to see the client's over time.

### 7.7.2. Relationship factors:

▪ Includes: fairness, learning opportunities, health, compatibility, and preferences.

1. **Fairness**:

- Means distributing the work so that no one gets enjoyable tasks, or all the unpleasant ones.
- Fairness also means an equitable amount of consideration for each member including consideration of requests for special assignment, or vacation time and equal sharing of rewards as well as hard work.

2. **Learning opportunities:**

- Although the assignment given must be within the capacity of the nurse, another factor to consider is that assignment stimulate motivation and learning.

3. **Health:**

- The health and well – being of team members are other factors to be considered … certain jobs are very stressful, so team members may need to be rotated through these assignments to keep stress at a tolerable level.
- Another concern is the effect of rotating shifts which disrupts body rhythms and increase fatigue, or special problems which must be considered in making assignments.

4. **Compatibility:**

- This does not mean that you try to put people who are alike together
- It means, mixing on this basis of skills chances for sharing and earning from others and can increase the overall efficiency of the team.

5. **Preference:**

- When they do not conflict with the other criteria for delegating responsibility, the preference of individual team members should also be considered.

**7.8. Assignment technique:**

1.Determine if the number of pts. In your ward and their illness state.

2. Check the number of nursing personnel available and their level of education, experience, abilities and skills.

3.Decide which pts need the most expert care.

4.Decide which nursing care delivery method you will use (case method – functional – primary – team)

- If you have a large number of pts. And an average number of nurses, select the team method because it provides the various abilities and skills of different levels of nursing personnel.

- If you have few very sick pts. And adequate nurses, select the case method the opportunity for nursing education is better and it increases satisfaction.

- If you are in the unfortunate situation, having many pts. And inadequate number of nurses, use the functional method… this allow for completion of essential tasks

5. Check to see that the entire work is covered.

6. Put your plan in a written form.

7. Post the written assignment in a definite place.

8. Explain orally to each member what his assignment is.

9. Make yourself available to answer any question.

10. Be ready for emergency.

# Chapter 8

# Teamwork and Team Building

## LEARNING OBJECTIVES

At the end of this chapter, the student should be able to:
1. Differentiate between team and group
2. Explain types of team
3. Discuss the advantages and disadvantages of teamwork
4. Demonstrate the five stages of team growth and building
5. Explain the 10 ingredients for a successful team

## 8.1 DEFINITION OF TEAMWORK

Teamwork is a group of people working together on the basis of:

1. Shared perceptions.
2. A common purpose.
3. A greed procedure.
4. Co-operation.
5. Commitment
6. Resolving disagreements openly by discussion.

***This will not happen automatically. Teamwork has to be managed, if it is to be effective.***

## 8.2 TYPES OF TEAMS

Teams are categorized according to:

**8.2.1. Composition**: skills, experience, and qualification of members. Ex. Nursing team composed of BSN – RN – PLN.

**8.2.2. Purpose**: purpose of the team. Ex. Surgical team – medical team)

**8.2.3. Function**: some team in community, in hospitals, and in emergency situation

**8.2.4. Leadership**: leadership in most team can be divided into 3 categories:

**(A) Designated leader:**

- Chosen by the team or by administration.
- Usually have clear role and responsibilities.
- Have more permanent position.

**(B) Emergent leader:**

- Assigned himself as leader.
- Usually have special traits that make him a leader.

**(C) Situational leader:**

- Temporary, according to the situation.

## 8.3. ADVANTAGES & DISADVANTAGES OF TEAMWORK

Leader team and working as a member of team presents a number of difficulties and challenges as well as benefits.

While they can't actually work in total isolation from other people in any health care setting. There are some people who prefer to work independently rather than interdependently

### 8.3.1. Advantages of Teamwork

**(A) Best use of skills**

- ♠ When people with complementary skills are brought together on team, each one is able to contribute own skills, experience and viewpoints to the task on hand.
- ♠ With the right combination of skills, a highly functional team can manage
- ♠ More complex situation, provide more comprehensive care and produce more creative ideas.

**(B) Coordination**

- ♠ Team work demands that team members communicate with each other …effective communication reduce duplication of efforts.

**(C) Synergy**

- ♠ When people work well together, combining their energies to complete a task ... has a synergetic effect in which each one stimulates and reinforce the others.

**(D) Flexibility**

- ♠ Team members help each other and substitute for each other in an emergency.

**(E) Support**

- ♠ Emotional support and actual assistance
- ♠ Manage anxiety and release tension

**(F) Increased commitment**

- ♠ Team's members who are involved in setting team's objectives and deciding how they will be met, are more committed to the successful completion of objectives.

**(G) Evaluation feedback**

- ♠ Quality of performance is more apparent

- ♠ Members can serve as valuable source of helpful feedback

**(H) Opportunity for growth**

- ♠ Including development of interpersonal and leadership skills
- ♠ Because members work closely together and share more of their experiences, this provides an opportunity to expend members knowledge and skills.

### 8.3.2. Disadvantages of Teamwork

**(A) Demands interpersonal skills**

- ♠ Effective team work requires a considerable number of interpersonal skills from members and leaders without these skills, team work can be frustrating, discouraging, energy consuming (improper selection of team members)
- ♠ Some people prefer to work independently.

**(B) Conflict**

- ♠ Differences in culture, experience, norms, evaluation, skills, status and pay are sources of conflict.

**(C) Time demands**

- ♠ Type of leader ship style affects the time needed for decision
- ♠ Democratic: long time, faster acceptance
- ♠ Autocratic: faster decision, longer time for acceptance

**(D) Reduced autonomy**

- ♠ The person who values independent work find it difficult to adapt; and work in a team.

**(E) Increased scrutiny**

- ♠ A person who works alone can hide mistakes and inadequacies

- ♠ Working in a team can be threatening to many people, especially to professionals who feel they should be perfect and not make mistakes

## 8.4. STAGES OF TEAM GROWTH

### Stage 1: Forming (immature team)

This is a stage of transition from individual to member status, and of testing the leader's guidance both formally and informally.

**Feelings involved:**

1. Excitement, anticipation and optimism
2. Suspicion fear and anxiety about the job ahead.

**Leader's Role in This Stage**

1. Help members get to know each other
2. Provide clear direction and purpose
3. Provide the information the team needs to get started.
4. Involving members in developing plans and clarifying roles

### Stage 2: Storming (fractionated team)

- ♠ The members are impatient about the lack of progress.
- ♠ Resistance to tasks and methods of work different from what each individual member is comfortable using.

**Feeling involved:**

1. Sharp fluctuation in attitude about the team's chance of success
2. Arguing, defensiveness, competition
3. Choosing sides and creation of perceived "Striking order, creating disunity, tension and jealousy.

4. A gap between initial expectations and reality of the situation

**Leader's Role in This Stage**

***Build a self-direction:***

1. Resolves issues of power and authority.
2. Develop and implement agreements about how decisions are made and who make them.
3. Encourage members to take on more responsibilities

## Stages 3: Norming (sharing team)

They accept the team, team ground rules (Norms), their role in the team and the individuality of other members

**Feeling involved:**

1. A sense of team cohesion.
2. Acceptance of membership in the team.
3. Relief that it seems everything is going to work out.

**Leader's Role in This Stage**

***Build cooperation***:

1. Fully utilize team member's skills
2. Encourage and acknowledge the respect members have for each other.
3. Encourage members to work together

## Stage 4: Performing (the effective team)

- ♠ The team has settled(stable) in relationships & expectations
- ♠ The team is now an effective cohesive unit
- ♠ They can performing & getting a lot of work done

**Leader's Role in This Stage**

***Build openness to chance:***

1. Update the team's methods and procedures to support cooperation
2. Help the team understand how to manage change
3. Monitor work progress and celebrate achievements

**Stage 5: mourning stage**

Ending with suffering

## 8.5. TEN INGREDIENTS FOR A SUCCESSFUL TEAM

### 1- Clarity in Team Goals

- ♠ Agree on the character or mission or works together to resolve disagreement.
- ♠ Has a clear vision and can progress rapidly towards its goals.

### 2- An Improvement Plan

- ♠ Has created a work plan
- ♠ Has a flowchart describing the steps of the work
- ♠ Knows what resources and training are needed

### 3- Clearly Define Roles

- ♠ Has formally designated roles
- ♠ Understand which role belongs to one person and which are shared
- ♠ Uses each one's talents and involves everyone in team activities

### 4- Clear communication

- ♠ Speak with clarity and directness

- ♠ Be concise, avoid long stories
- ♠ Listen actively, explore
- ♠ Avoid interrupting and talking when others are speaking.

**5- Beneficial Team Behaviors**

- ♠ Initiate discussions
- ♠ Seek information and opinions
- ♠ Clarify ideas
- ♠ Test for agreement
- ♠ Be creative in resolving differences
- ♠ Try to ease tension in the group
- ♠ Express the group feelings and ask others to check the impression
- ♠ Get the group to agree on standards
- ♠ Praise and correct others with equal fairness, accept both
- ♠ praise as well as complains

**6- Well defined decision Procedures**

- ♠ Decide how the decisions will be made
- ♠ Explore important issues by polling Test for agreement
- ♠ Use data as the basis of decisions

**7- Balanced Participation**

- ♠ Have reasonably balanced participation with all members contributing to most discussions
- ♠ Build on members natural style of participation

**8- Established Ground Rules**

- ♠ Have open discussions about ground rules, what behavior are acceptable and unacceptable
- ♠ Get the group to agree on standards
- ♠ Refer to documentation and data
- ♠ Praise and correct others with equal fairness, accept both praise as well as complaints

**9- Awareness of the Group Process**

- ♠ Be sensitive to non- verbal communication
- ♠ See, hear and feel the team dynamics
- ♠ Comment and intervene to correct a team process problem
- ♠ Contribute equally to group process and meeting content

**10- Use of Scientific Approach**

- ♠ Ask to see data before making decisions
- ♠ Use basic statistical tools to investigate and gather and analyze data
- ♠ Pick up for root causes of problems
- ♠ Seek permanent solutions rather than rely on quick fixes

# Chapter 9

# Roles and Functions in Directing

## LEARNING OBJECTIVES

*At the end of this chapter, the student should be able to:*

1. Define directing and motivation
2. List the types of motivation
3. Outline the different theories of motivation
4. Discuss the different motivation theories
5. Explain how to create a motivating climate

## 9.1. INTRODUCTION

The fourth phase of the management process is directing. This phase also may be referred to as coordinating or activating. This is the "doing" phase of management, requiring the leadership and management skills necessary to accomplish the goals of the organization. Managers direct the work of their subordinates during this phase.

Components of the directing phase include creating a motivating climate, establishing organizational communication, managing conflict, facilitating collaboration, and negotiating.

In planning and organizing, managers attempt to establish an environment that is conducive to getting work done. In directing, the manager sets those plans into action.

This chapter focuses on creating a motivating climate as a critical element in meeting employee and organizational goals. The amount and quality of work accomplished by managers directly reflect their motivation and that of their subordinates.

***To understand this subject, let us explore the following questions:***

*1- Why are some managers/ employees more motivated than others?*

*2- How do motivated managers affect their subordinates?*

*3- What should the manager do with the employee who is demotivated?*

## 9.2. DIRECTING

In order for the nurse manager to guide and direct staff members, he must first understand the nature of human behavior and the reasons behind that behavior.

To understand another person, you must know how he feels about himself. Personal self-image will influence behavior. A person's self-concept is acquired from childhood experiences and from the feedback he receives for his achievements. A person's behavior is purposeful, and the manager needs to be perceptive in understanding why staff members are behaving as they are.

The successful manager learns the reasons behind the behavior and takes the necessary steps to correct undesirable behavior. Individuals attempts to fulfill their wants, drives and needs. The word motive implies action to satisfy a need.

## 9.3. MOTIVATION

Motivation is the force within the individual that influences or directs behavior. Because motivation comes from within the person, managers cannot directly motivate subordinates.

The humanistic manager can create an environment that maximizes the development of human potential. Management support, collegial influence, and the interaction of personalities in the work group can have a synergistic effect on motivation.

The leader–manager must identify those components and strengthen them in hopes of maximizing motivation at the unit level. All human beings have needs that motivate them.

The leader focuses on the needs and wants of individual workers and uses motivational strategies appropriate for each person and situation. Leaders should apply techniques, skills, and knowledge of motivational theory to help nurses achieve what they want out of work.

At the same time, these individual goals should complement the goals of the organization. The manager bears primary responsibility for meeting organizational goals, such as reaching acceptable levels of productivity and quality.

The leader–manager, then, must create a work environment in which both organizational and individual needs can be met. Adequate tension must be created to maintain productivity while encouraging subordinates' job satisfaction. Thus, while the worker is achieving personal goals, organizational goals are being met

## 9.3. TYPES OF MOTIVATION

### 9.3.1. Intrinsic motivation:

Comes from within the individual, driving him to be productive. To be intrinsically motivated at work, the nurse must value job performance and productivity.

*The intrinsic motivation to achieve is directly related to:*

a. Person's level of aspiration.

b. Person's believe that improved performance led to outcomes congruent with their value.

c. Increased effort will lead to improved performance and increase productivity and thus, the required energy expenditure will be worth the cost.

### 9.3.2. Extrinsic motivation:

Enhanced by job environment or external rewards. The reward occurs after the work has been completed.

Although all individuals are intrinsically motivated to some degree, it is unrealistic for organization to assume that all workers have adequate levels of intrinsic motivation to meet organizational goals.

*THUS, organization must provide a climate that stimulates both extrinsic and intrinsic drives. Because every human being is unique and have different needs, so every person is motivated differently*.

The intrinsic motivation to achieve is directly related to a person's level of aspiration. Extrinsic motivation is motivation enhanced by the job environment or external rewards. Because motivation is so complex, the leader faces tremendous challenges in accurately identifying individual and collective motivators.

## 9.4. MOTIVATIONAL THEORY

### 9.4.1 Abraham Maslow

Continued focus on human motivation did not occur until Abraham Maslow's work in the 1950s. Most nurses are familiar with Maslow's hierarchy of needs and theory of human motivation.

Maslow (1970) believed that people are motivated to satisfy certain needs, ranging from basic survival to complex psychological needs, and that people seek a higher need only when the lower needs have been predominantly met. Maslow's work contributed to the recognition that people are motivated by many needs other than economic security

*Because of Maslow's work, managers began to realize that people are complex beings, and rather than just being motivated by economics, they many needs motivating them at any one time.*

It also became clear that motivation is internalized and that if productivity is to increase, management must help employees meet lower-level needs. The shifting focus on what motivates employees has tremendously affected how organizations value workers today.

### 9.4.2. Skinner

Reinforcement Theory (Operant Conditioning) skinner. Another theorist who contributed to understanding of motivation, dissatisfaction and productivity. Skinner's research on operant conditioning and behavior modification demonstrated that people can be conditioned to behave in a certain way based on a consistent reward or punishment.

Behavior that is rewarded will be repeated and behavior that is punished will be decreased or stopped.

### 9.4.3. Herzberg

Frederick Herzberg (1977) believed that employees can be motivated by the work itself and that there is an internal or personal need to meet organizational goals. He believed that separating personal motivators from job dissatisfiers was possible. He hygiene or maintenance factors and motivator factors was called the Motivation–Hygiene theory or Two Factor theory.

Herzberg maintained that motivators or job satisfiers are present in work itself; they give people the desire to work and to do that work well. Hygiene or maintenance factors keep employees from being dissatisfied or demotivated but do not act as real motivators

It is important to remember that the opposite of dissatisfaction may not be satisfaction. When hygiene factors are met, there is a lack of dissatisfaction, not an existence of satisfaction. Likewise, the absence of motivators does not necessarily cause dissatisfaction

***For example**, salary is a hygiene factor. Although it does not motivate in itself, when used with other motivators such as recognition or advancement, it can be a powerful motivator. If, however, salary is deficient, employee dissatisfaction can result. Some argue that money can truly be a motivator, as*

*evidenced by people who work insufferable hours at jobs they truly do not enjoy. Some theorists would argue that money in this case might be taking the place of some other unconscious need.*

**Display 18.2 Herzberg's Motivators and Hygiene Factors**

| Motivators | Hygiene Factors |
|---|---|
| Achievement | Salary |
| Recognition | Supervision |
| Work | Job security |
| Responsibility | Positive working conditions |
| Advancement | Personal life |
| Possibility for growth | Interpersonal relationships and peers |
| | Company policy |
| | Status |

## 9.4.4. McClelland

**David** McClelland (1971) examined what motives guide a person to action, stating that people are motivated by three basic needs: **achievement, affiliation, and power.**

**Achievement-oriented** people actively focus on improving what is; they transform ideas into action, wisely, taking risks when necessary.

**Affiliation-oriented** people focus their energies on families and friends; their overt productivity is less because they view their contribution to society in a different light from those who are achievement oriented. People with strong need for affiliation are likely to prefer a job that entails a lot of social interaction.

**Power-oriented** people are motivated by the power that can be gained

## 9.4. 5. Gellerman

Saul Gellerman (1968), another humanistic motivational theorist, has identified several methods to motivate people positively.

One such method**, stretching**, involves assigning tasks that are more difficult than what the person is used to doing. Stretching should not, however, be a routine or daily activity but an activity used to help the employee grow.

Another method, **participation,** entails actively drawing employees into decisions affecting their work.

*The challenge of "stretching" is to energize people to enjoy the beauty of pushing themselves beyond what they think they can do.*

## 9.4.6. McGregor

Douglas McGregor (1960) examined the importance of a manager's assumptions about workers on the intrinsic motivation of the workers.

These assumptions, which McGregor labeled
*Theory X* and *Theory Y* (depicted in Display 18.3), led to the realization in management science that how the manager views, and thus treats, the worker will have an impact on how well the organization functions.

**Display 18.3 McGregor's Theory X and Theory Y**

| Theory X Employees | Theory Y Employees |
|---|---|
| Avoid work if possible | Like and enjoy work |
| Dislike work | Are self-directed |
| Must be directed | Seek responsibility |
| Have little ambition | Are imaginative and creative |
| Avoid responsibility | Have underutilized intellectual capacity |
| Need threats to be motivated | Need only general supervision |
| Need close supervision | Are encouraged to participate in problem solving |
| Are motivated by rewards and punishment | |

**Theory X assumes that:**

Most people would rather be directed than assume responsibility for creative problem solving.

They find work distasteful and are motivated primarily by physical and security needs

They must be managed through close supervision, seek rewards in money and other fringe benefits and punishment.

**Theory Y assumes that:**

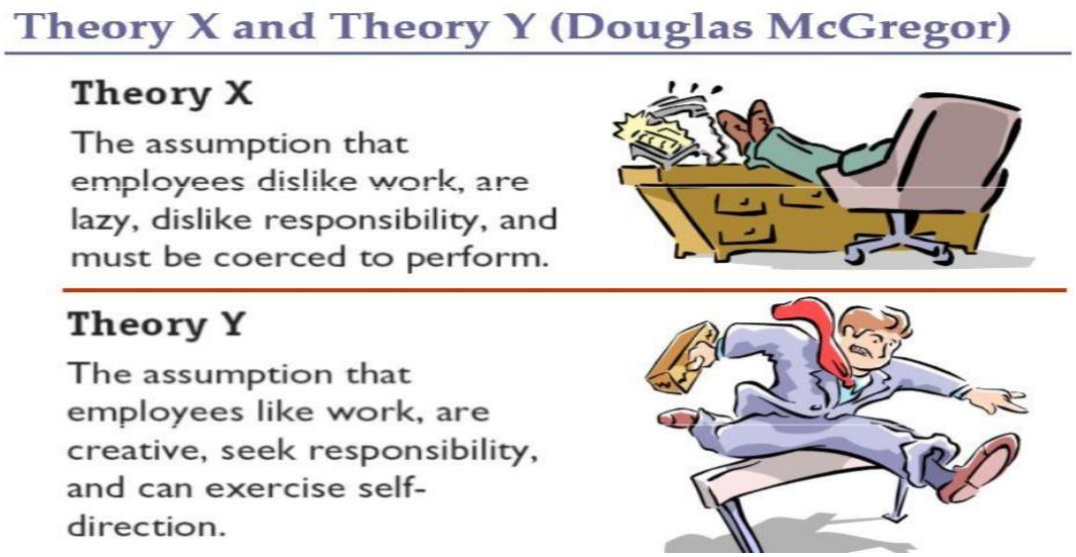
People are basically reliable and naturally enjoy work if conditions are favorable.

Can be self-directed.

Can be creative and welcome opportunities make contributions.

Can be motivated by exposure to less external control and progressively more self-control.

***Theory Y is not a "better" management style than Theory X; the style which is "best" depends on the variables inherent in a given situation.***



## 9.5.HOW CAN A NURSE MANAGER CREATE A MOTIVATING CLIMATE?

*The nurse manager can create a motivating climate by:*

**A- Positive reinforcement:**

The most powerful motivator a manager can use.

Some approaches to effective reinforcement includes:

**1-** Positive reinforcement must be specific or relevant to a particular performance

**2-** Positive reinforcement must have immediacy by occurrence as close to the event as possible.

**3-** The reward feedback must be achievable.

**4-** Rewards should be unpredictable and intermittent. if rewards are given routinely, they tend to lose their value.

**B- By being a positive and enthusiastic role model in the clinical setting:**

- Employees frequently gauge their job security and their employer's satisfaction with job performance by the expression that see on their manager's face.

**C- Mentor future managers:**

- KOTTER (1985) stated that "managers must be mentors, sponsors, coaches and role model, if new employees are to succeed in their roles.

## 9.6. STRATEGIES TO CREATE A MOTIVATING CLIMATE:

In addition to previous points, the manager must use the following strategies to create a motivating climate:

1) Have clear expectations from nurses and communicate these expectations effectively.
2) Be fair and consistent in dealing with all nurses.
3) Be a firm decision-maker … not hesitated.
4) Develop the concept of teamwork, develop group goals and projects that will build a team spirit.
5) Integrate the staff needs and wants with organizational interests and purposes.

6) Know the uniqueness of each nurse (as a person), and let each one understands that you know his uniqueness.
7) Remove traditional blocks between nurses and the work to be done.
8) Provide experiences that challenge or (stretch) the nurses and allow opportunity for growth.
9) When appropriate, let subordinates participate in decision-making.
10) Give nurses recognition and credit
11) Make sure nurses understand the reasons behind decisions and actions.
12) Reward desirable behavior.
13) Create a trustful and helping relationship with employee

***Examples of motivators**: salary boluses – paid off days – enrolled in further studies – promotions – certificates – best employee of the year.*

# CHAPTER 10

# COMMUNICATION

## LEARNING OBJECTIVES

*At the end of this chapter, the student should be able to:*

1. Define verbal communication and non-verbal communication

2. Explain the communication dimensions and communication process

3. List down the components of communication process using a diagram direction

4. Discuss the basics for good communication

5. List down common blocks to communication and discuss on how to improve them

6. Use through examples the common patterns of communication networks.

## 10.1. COMMUNICATION

Communication: transmitting a message with the expectation of some kind of response. This can be interpersonal or intrapersonal.

Communication is a process that allows organisms to exchange information by several methods. Exchange requires feedback.

## 10.2. COMMUNICATION DIMENSIONS

Communication is usually described along a few major dimensions:

▪ Content (what type of things are communicated)

▪ Source/ Sender/Encoder (by whom)

▪ Form (in which form)

▪ Channel (through which medium)

▪ Destination/Receiver/Target/Decoder (to whom)

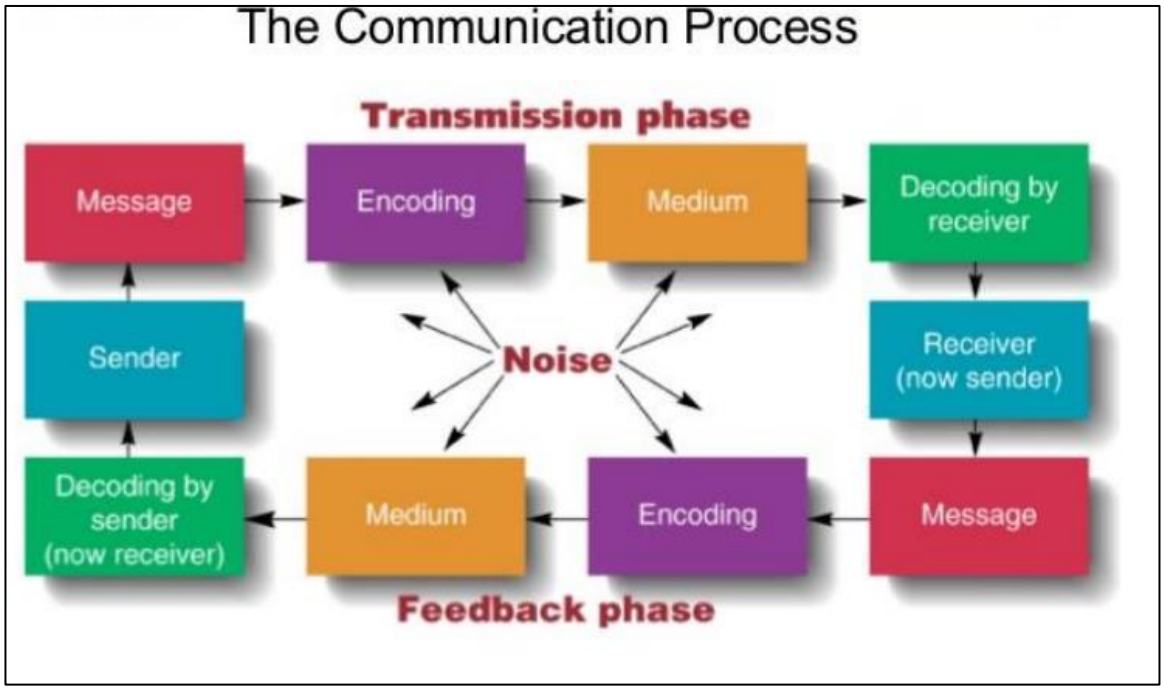
▪ Purpose

## 10.3. THE COMMUNICATION PROCESS

▪**Thought**: First, information exists in the mind of the sender. This can be a concept, idea, information, or feelings.

▪ **Encoding**: Next, a message is sent to a receiver in words or other symbols.

▪ **Decoding**: lastly, the receiver translates the words or symbols into a concept or information that he or she can understand.




## 10.4. ORGANIZATIONAL COMMUNICATION OBJECTIVES

- ♠ An open communication climate.
- ♠ Accurate information.
- ♠ Competition and cooperation rather than conflict.
- ♠ The “Chain” Communication Structure
- ♠ The “Circle” Communication Structure

## 10.5. LEVELS OF COMMUNICATION

| | |
|---|---|
| 1. Intrapersonal | A. Occurs within an individual |
| 2. Interpersonal | B. One-to-one interaction between two people |
| 3. Small group | C. Interactions with a small number of people |
| 4. Public | D. Interaction with an audience |
| 5. Electronic | E. Using technology to create ongoing relationships with patients |

## 10.6. TYPE OF COMMUNICATION

### 10.6.1. Verbal Communication

- Pace and intonation
- Simplicity
- Clarity and brevity
- Congruence
- Timing and relevance
- Adaptability
- Credibility
- Humor

### 10.6.2. Nonverbal Communication

- **Body language: Gestures**, movements, use of touch
- **Essential skills: observation, interpretation**
    - Personal appearance
    - Posture and gait
    - Facial expression of self, others; eye contact
- **Gestures**

- Cultural component

### 10.6.3. Electronic Communication

**Advantages**

Fast
Efficient
Legible
Improves communication, continuity of care

**Disadvantages**

Client confidentiality risk
Socioeconomics

**Do not use e-mail**

Urgent information
Jeopardy to client's health
Highly confidential information
Abnormal lab data

**Other guidelines**

Agency-specific standards and guidelines
Part of medical record
Consent, identify as confidential

## 10.7. COMMUNICATION SYSTEM

▪ Downward Communication.

▪ Upward Communication.

▪ Lateral Communication.

▪ Diagonal Communication.

▪ Grapevine Communication.

### 10.7.1. Downward communication:

▪ Formal communication consists of messages sent from superiors to subordinates.

▪ Instructions or directions concerning job-performance.

▪ Information about organizational procedures and policies

▪ Feedback to the subordinates concerning job performance

▪ Information to assist in of work tasks.

▪ Low employee satisfaction

▪ Downward communication is limited.

▪ Downward communication must accomplish the following:

- ✓ Demonstrate empathy.
- ✓ Be accurate
- ✓ Be definite.
- ✓ Consider selective perception.
- ✓ Provide for feedback
- ✓ Avoid message overload

### 10.7.2. Upward communication.

▪ Upward communication: the flow of messages from the lower levels of the organization to the upper levels.

▪ It most consists of information such as feedback of lower-level operations (reports of production).

▪ Upward is important work-related decisions.

▪ Upward communication involves complaints and suggestions for improvement

▪ High employee's satisfaction

▪Successful upward communication requires the following:

- ✓ Frequent face-to-face contact
- ✓ Recognition of and rewards for accomplishment
- ✓ The ability to listen
- ✓ Action.

### 10.7.3. Lateral communication

▪ This is the flow of communication between people who are on the same level in an organization.

▪ It requires co-worker's co-ordination to accomplish a goal.

▪ Lateral communication can also occur between two or more departments in an organization.

▪ Lateral communication helps develop interpersonal relationships.

▪ Lateral communication has three formal functions:
1. Coordination.
2. Problem solving.
3. Conflict resolution
▪ Lateral communication tends to be faster, more accurate, and less threatening than vertical communication.

### 10.7.4. Diagonal communication

▪ It occurs between individual or departments that are not in the same

level of the hierarchy.

▪ Informal

▪ The grapevine

▪ Rapid and fast

▪ Grapevine has three main characteristics.

1. it is not controlled by management.

2. it is perceived by most employees as being more believable and

reliable than formal channel issued by top management.

3. it is largely used to serve the self-interests of the people within it.

## 10.8. BARRIERS TO COMMUNICATION

### 10.8.1. Culture, background, and bias

We allow our past experiences to change the meaning of the message. Our culture, background, and bias can be good as they allow us use our past experiences to understand something new, it is when they change the meaning of the message then they interfere with the communication process.

### 10.8.2. Noise

Equipment or environmental noise impede clear communication. The sender and the receiver must both be able to concentrate on the messages being sent to each other.

### 10.8.3. Ourselves

Focusing on ourselves, rather than the other person can lead to confusion and conflict. The "Me Generation" is out when it comes to effective communication. Some of the factors that cause this are defensiveness (we feel someone is attacking us), superiority (we feel we know more than the other), and ego (we feel we are the center of the activity).

### 10.8.4. Perception

If we feel the person is talking too fast, not fluently, does not articulate clearly, etc., we may dismiss the person. Also, our preconceived attitudes affect our ability to listen. We listen uncritically to persons of high status and dismiss those of low status.

### 10.8.5. Message

Distractions happen when we focus on the facts rather than the idea. Our educational institutions reinforce this with tests and questions. Semantic distractions occur when a word is used differently than you prefer. For example, the word chairman instead of chairperson, may cause you to focus on the word and not the message.

### 10.8.6. Environmental

Bright lights, an attractive person, unusual sights, or any other stimulus provides a potential distraction.

### 10.8.7. Smothering

We take it for granted that the impulse to send useful information is automatic. Not true! Too often we believe that certain information has no value to others or they are already aware of the facts.

### 10.8.8. Stress

People do not see things the same way when under stress. What we see and believe at a given moment is influenced by our psychological frames of references - our beliefs, values, knowledge, experiences, and goals.

## 10.9. ASSERTIVENESS

Assertive style of behavior is to interact with people while standing up for your rights

▪ **Assertive people**: Assertive people have the following characteristics

1. They feel free to express their feelings.
2. They know their rights.
3. They have control over their anger.

# CHAPTER 11

# EFFECTIVE DELEGATION AND SUPERVISION

*"Delegation is a powerful tool when used correctly can create an effective and safe environment in which staff and patients work together to achieve positive outcomes."*

## LEARNING OBJECTIVES

At the end of this chapter, the student should be able to:

1. Define delegation, assignment and supervision
2. List the benefits of delegation
3. Discuss the five rights of delegation
4. Explain how to develop safe delegation practice
5. Identify the barriers of delegation, and how to overcome these barriers
6. Identify the barriers for delegator and delegatee

## 11.1. DELEGATION

▪ Transfer of responsibility for the performance of an activity from one individual to another while retaining accountability for the outcome.

▪ Transfer to competent individuals the authority to perform selected nursing task in a selected situation, it relies on trust and the goal of delegation is workload distribution.

▪ Sharing responsibility and authority with subordinates and holding them accountable for performance

## 11.2. THE BENEFITS OF DELEGATION ARE:

▪ Delegation encourages the initiative of subordinates and creates room for them to grow.

▪ It frees managers for other priorities, including planning, organizing and directing.

▪ Controlling the organization itself.

## 11.3. THE FIVE RIGHTS OF DELEGATION

### 11.3.1. Right Task

▪ Potential for harm? great or small

▪ Complexity of task? simple or difficult

▪ Problem solving needed? minimal or maximum

▪ Unpredictability of outcome? stable or unstable

▪ Level of interaction required with client? simple or complex

### 11.3.2. Right Circumstances

▪ Delegating a task under the right circumstances includes

▪ Considering the condition and acuity of the patient

▪ An awareness of the staff mix.

▪ Organization documents – the job descriptions, policies, and procedures within the organization will guide to whom and what can be delegated.

▪ Setting, available resources, and other relevant factors.

### 11.3.3. Right Person

▪ Nurse Practice Act – each state has a nurse practice act that will define and determine what task can be delegated to what level of personnel.

▪ Determines the scope of practice for each position

▪ Issues standards of care that guide practice

▪ Organization guidelines – each organization has statements that guide practice.

▪ Mission statement

▪ Policies and procedures (determine what can be done and what level of personnel can carry out the task)

▪ Job descriptions – identifies job responsibilities

▪ The right person is delegating the right task to the right person to be performed on the right patient or consumer

### 11.3.4. Right Communication

▪ Clear concise description of the task, including its objectives, limits and expectations

▪ This communication should include the:

- Task with clear expectations of what is to be done, why it is being done, and when the task should be completed
- Correct patient
- Information that should be reported back to the nurse and when that information should be given to the nurse
- Expected outcome of the task

### 11.3.5. Right Supervision

▪ Checking on the progress of the task

▪ Checking on the outcomes once the task is performed

▪ Obtaining feedback about the completed task from the delegatee

▪ Evaluate the effectiveness of the delegation with the delegatee

▪ Giving feedback to the delegatee is important. To create an environment where feedback is seen as constructive, consider the following:

* Timing of the feedback –give the delegatee feedback as soon as possible after the task is completed
* Focus on the behavior/performance of the task
* Provide clear feedback addressing the completion of the task
* Provide feedback privately with the delegatee
* Ask for feedback from the delegatee to clarify any concerns that person may have as well

## 11.4. SUPERVISION

Is Influencing the outcome of an individual's performance by directing, guiding, and monitoring the activity and providing support to enhance the performance

**Right Supervision (watching, directing, & correcting**)

▪Evaluation with:

- Completed with no correction, & praise
- Completed with minimal correction, & praise
- Incomplete with more practice, re-performance, encouragement
- Incomplete with more education/practice, re-performance, and encouragement.

▪ Document progress/evaluation.

## 11.5. DELEGATOR

▪ As the delegator, the RN must

▪ Have a view of the big picture in the care of the assigned patients

▪ Select the appropriate activities to delegate

▪ Select the appropriate staff to carry out the activities

▪ Clearly communicate the expectations and required follow up

▪ Follow up periodically while the task is being completed

▪ Evaluate and provide feedback on the effectiveness of the delegation to staff

## 11.6. DELEGATEE

▪ The delegatee also has responsibilities including

▪ Accepting the delegated activities as directed by the manager

▪ Give report to manager as requested on the delegated task

▪ Give feedback to delegator on the effectiveness of the delegation process

## 11.7. WHAT IS DELEGATION? WHAT IS NOT DELEGATION?

| Delegation is | Delegation is not |
|---|---|
| **Two-way relation.** | A relation from higher to lower. |
| **A continuing process.** | Something we do only once. |
| **Giving authority to subordinates.** | Using authority. |
| **Letting subordinate use personal initiative** | Taking decisions for subordinates. |
| **Permitting subordinate to control** | Distributing workloads only. |
| **Encouraging initiative and team work** | Controlling subordinate |
| **Giving coaching** | Passing one's responsibilities on to others. |
| | Directing. |

## 11.10. DEVELOPING SAFE DELEGATION PRACTICE

### 11.10.1. Know the patient

- ♠ A nursing assessment must be completed before delegation- knowing level of care required by the patient, considering the clinical, physiological, emotional, cognitive, & spiritual status.
- ♠ Is the patient condition stable? The more stable the patient, the more likely delegation is to be safe
- ♠ What is the potential for change in the patient condition as a result of the delegated task? (high risk)

### 11.10.2. Know the staff member

- ♠ Before the task is delegated, the delegatee must have skills & knowledge necessary to perform the task, as evidenced by the person job description, training program and document of competencies.
- ♠ Experience and past job performance should be considered
- ♠ Is the staff member knowledgeable and trained to perform the task
- ♠ Select the right person for the right task

### 11.10.3. Know the task to be delegated

- ♠ Task must be in scope of practice.
- ♠ Routine, standardized tasks that are performed according to a standard and unchanging procedure and have predictable outcomes are the safest to delegate
- ♠ Complex tasks or activities that pose high risk for patient complications or unpredictable outcomes should be examined closely before delegation is considered

### 11.10.4. Explain the task and expected outcomes

- ♠ The manager explains the delegated task, what must be done, and the expected outcomes.
- ♠ Failure to effectively communicate what is expected may result in unsatisfactory performance, error, and possible harm to the patient.
- ♠ Directions should be provided in clear, concise manner.
- ♠ The delegated task is acceptable only when the staff member understands the task and is adequately prepared to carry it out.

### 11.10.5. Expect responsible action from the delegatee

- ♠ When the staff member accepts and understand the task, he or she should then be allowed to perform task
- ♠ The staff member becomes responsible for his or her actions and is obligated to complete the task as mutually agreed.

- ♠ The manager should provide appropriate supervision.
- ♠ The manager should expect responsible actions, give authority and retain accountability

### 11.10.6. Assess and supervise job performance

- ♠ Supervising job performance provides a mechanism for feedback and control.
- ♠ Job performance is assessed by making frequent rounds, observing, and communicating (ask about progress and determine whether there are any questions or concern)
- ♠ Supervise in a positive and supportive manner to reassure the staff member that his or her work is important and appreciated.
- ♠ Poor performance must be documented and reported to the nurse manager. Never ignore poor performance, when a mistake is made, use it as learning opportunity for the staff member involved

### 11.10.7. Evaluate and follow up

- ♠ Once the task is complete, evaluate the staff members performance and reassess the patient to ensure that the expected outcomes were achieved.
- ♠ Follow up with any interventions that may be required based on patients care outcomes or the delegatee's job performances.
- ♠ Review and document the skills that were learned.

## 11.11. CHALLENGES WITH DELEGATION

- ♠ Delegatee refuses to accept the delegated task.
- ♠ Delegator – go rapidly through the 5 rights
- ♠ Ask the delegatee why the request is being refused
- ♠ May be due to
    - Fear – afraid cannot complete the task correctly & safely

- Told not to do by others – other staff have told the delegatee never to carry out this task
- Lack confidence – the delegatee may be new or lack the confidence to complete the task
- Overwhelmed – may be concerned about the workload already assigned

## 11.12. BARRIERS TO DELEGATION

- ♠ Lack of confidence and trust in subordinates.
- ♠ The belief that the manager himself can do it only.
- ♠ Low self-confidence, insecurity, fear that subordinate might subsequently jeopardize his position.
- ♠ Prestige and power consciousness.
- ♠ Lack of skills in communication.

## 11.13. BARRIER ON THE PART OF DELEGATE

- ♠ Lack of technical competence and skills required to carry out a task.
- ♠ Lack of willingness to accept responsibility.
- ♠ Lack of initiative.
- ♠ lack of self-confidence.
- ♠ Fear of loss of power.
- ♠ Lack of control.
- ♠ Inadequate reward.
- ♠ Have more work than they can do.

## 11.14. OVERCOMING THIS BARRIER

- ♠ The delegator must accept that part of his activity is done by others.

- The delegator emphasizes that the behavior of people is the key to productivity and success depends on motivated and skilled employee who are committed to organizational objectives.
- Recognize what is the causes that prevent delegation.

# CHAPTER 12
# DEALING WITH CONFLICT

## LEARNING OBJECTIVES

*At the end of this chapter, the student should be able to:*

1. Define the conflict.
2. Determine the levels, causes and important of conflict
3. Discuss school of thoughts regarding conflict
4. Explain the conflict process
5. Determine the outcomes of the conflict
6. Discuss the five-conflict approach
7. Explain the conflict management skills

## 12.1. DEFINITION OF CONFLICT

Conflict is a disagreement that occurs between two or more parties who believe that what each want is incompatible with what the other wants.

Conflict is A condition that exists anytime two or more people disagree.

## 12.2. LEVELS OF CONFLICTS

**Personal level:** interpersonal, between persons.
**Group level:** between groups inside the organization
**Organizational level:** between two organizations.

## 12.3. CAUSES OF CONFLICTS

- Differences in goals between parties.
- Increasing complexity and specialization of health care institutions.
- The rising expectations of those delivering and receiving health care.
- Increase the size of the group.
- The changing role of the nurse.
- Increase competitions among health care institutions.

- Increase regulations and rules.

## 12.4. IMPORTANCE OF CONFLICT

1. The presence of conflict does not necessarily mean a negative process is occurring should be viewed positively as it can lead to growth, energy and creativity generating new ideas.
2. Conflict is viewed a not only inevitable, but as natural condition and necessary if people and organizations are to change for best.
3. It is important for nurse manger to understand of conflict, mean to resolve it, for their units to perform effectively.

## 12.5. SCHOOL OF THOUGHTS REGARDING CONFLICT

### 12.5.1. Traditional view: the early approach.

1) Conflict is bad (negative)
2) Must be avoided
3) Indicates malfunction in the group.

### 12.5.2. Behavioral view:

1) Conflict is natural and inevitable outcome in any group.
2) It has the potential to be positive force in determining group performance.
3) Cannot be eliminated.

### 12.5.3. Interactionist view: most recent one

1) Argues that some conflict is absolutely necessary for any group to perform effectively.
2) It encourages conflict on the grounds that a harmonious, peaceful, cooperative group is likely to become static and nonresponsive to needs for change … So, the major contribution of this approach is encouraging leaders to maintain an ongoing minimal level of conflict enough to keep the group a live, self-critical and creative.

## 12.6. CONFLICT PROCESS

### 12.6.1. Potential opposition:

*Presence of conditions that creates opportunities to raise, it includes:*

**a. Communication difficulties:**

- Misunderstand and noise in the communication channel led to retardation of collaboration.

**b. Structure:**

- Increase group size … increase conflict
- Routines, specialization and standard in the task.

**c. Personal variables:**

- Individuals who authoritarian and who demonstrates, leads to potential conflict.

### 12.6. 2. Cognitive and personalization:

- If the conditions cited in stage one generates frustration then the potential for opposition become realized.
- There is disagreement with colleagues.
-

### 12.6. 3.Behavior:

- Members in the group engages in actions that attainment of another's goals.
- That action must be intended, that is, there must be a known to frustrate others.
- At this level the conflict is out in the open.
- Behavior ranges from indirect, to direct, aggressive, violent and uncontrolled struggle.

## 12.7. OUTCOME

### a. Functional: (positive/constructive)

- At low levels of conflict.
- Improves quality of decisions … stimulates creativity
- Encourage interest and curiosity among group members.

**Positive outcome includes:**

1. Better ideas produced.

2. People are forced to search for new approaches.
3. Long standing problems are brought to the surface and resolved.
4. Clarification of individual views.
5.

**b. Dysfunctional (negative / destructive)**

- ❖ At high levels of conflict.
- ❖ Results in break in communication and misunderstanding.
- ❖ Decrease group performance and effectiveness.

**c. Negative out comes include:**

1. Some people feel defeated and demanded.
2. Increase distance between people/group.
3. A climate of mistrust and suspension.
4. Increase resistance.
5. Increase voluntary labor turnover (V L T)

## 12.8. CONFLICT SOLVING APPROACHES

*Five approach using 2 dimensions*.

**A. Cooperativeness:**

the degree to which on party attempts to satisfy the other party's concerns.

**B. Assertiveness:**

the degree to which one party attempts to satisfy its own concerns

### 12.8.1. The Five Approaches

**1.Competition:** ( assertive + uncooperative)

**2.Collaboration:** (assertive + cooperative)

**3.Avoidance:** (unassertive + uncooperative)

**4.Accommodation:** (unassertive + cooperative)

**5.Compromise:** (mid-way between assertive + cooperative )

**1. Competition (assertive + uncooperative)**

One party seeks to achieve is goals regardless of the impact on the other parties. Each party will use its power base to again a victory in its favor.

**WIN – LOSE approach**

**2. Collaboration (assertive + cooperative)**

Each party desire to satisfy the concerns of all parties. The aim is solving the problem and clarifying the differences.

**WIN – WIN approach**

**3. Avoidance (unassertive + uncooperative)**

One party react by withdrawing or suppressing the conflict

**LOSE – LOSE approach**

**4. Accommodation (unassertive + cooperative)**

One party is willing to be self-scarifying for the other to maintain the relationship.

**LOSE – WIN approach.**

**5. Compromise (mid-way between assertive + cooperative)**

Each party in the conflict must give up something. Sharing occurs, results in compromise outcome Negotiations between parties leads to any agreement.

**NO CLEAR WINNER OR LOSER**

***You Can Think Before You Act***

***It's likely that a person employs more than one style, depending on the situation, but usually one style dominates.***

***Certain styles may be appropriate for certain situations.***

***Managing conflict constructively depends in large measure on clear, open, and honest communication***

## 12.9. CONFLICT MANAGEMENT SKILLS

1) Communications Fundamentals
2) Effective Listening and Speaking
3) Issues vs. Interests vs. Positions
4) Asking Effective Questions

### 12.9. 1. Communications Fundamentals

- Find a good time and place to talk.
- Discuss the problem - Clarify issues
- Get all the facts
- Use active listening
- Use "I" messages
- Focus on the problem, not the person
- Avoid communication blockers
- Present information in way to encourage thought and dialogue.
- Confirm understanding of what you have said.
- Indicate if you want more information.
- Prepare prior to giving difficult, complicated, or upsetting information.
- Avoid words that might be offensive or hurtful.

### 12.9.2.Effective Listening

**WHAT TO DO**

- Listen for feelings as well as facts.
- DO NOT INTERRUPT - LET THEM FINISH
- Make eye contact.
- Check for words or phrases you might not understand.
- Summarize or paraphrase accurately.
- Create an atmosphere for listening.

- Have the patience to 'get it right'!

**WHAT TO AVOID**

- Making judgments
- Doing anything else while listening
- Interrupting or completing the speaker's sentence
- Assuming you know what, the speaker is going to say
- Offering advice or solutions

**WHAT TO DO**

- Look directly at the person to whom you are speaking.
- Be clear about what you INTEND –what you mean.
- Be clear about what you FEEL -if appropriate.
- Be organized.
- Finish your thoughts.
- Provide examples as needed.

**WHAT TO AVOID**

- Using words or tones that are offensive
- Assuming the listener knows what, you are talking about
- Trying to cover too much information at once
- Assuming the listener shares your values or interests

## Examples of Effective Speaking

1. **Tone and Words:** *"This report is NEVER on time." vs. "This report is frequently late."*

2. **Specific:** *"This report covers the nursing home expenses for the second quarter."*
3. **Limit the information:** *"I want to discuss the three items today and the balance next week."*
4. **Share values or interests:** *I am interested in your feedback because I need to complete next year's budget this week.*

## Asking Effective Questions

- Basic Principle –The FORM of the question can determine the:

**Length of the response**

- –Short
- –Long

**Content of the information**

- –Facts
- –Description
- –Analysis

## Effective Questions

- ∴ **HOW** you **ASK** the question can influence the RESULT of the interaction.
- ∴ Avoid **WHY** and **LEADING** questions.
- ∴ Selectively use **CLOSED** and **OPEN-ENDED** questions.

## Flexibility

- ∴ be able to adjust to any situation
- ∴ do not try to handle the situation alone - use your partner(s)

- understand you are not going to please everyone - agree to disagree - this is a two-way street, but the official has final say

## 12.10. STEPS TO FOLLOW

1. Defining the problem
2. Generating possible solutions (brainstorming).
3. Evaluating and testing the various solutions.
4. Deciding on mutually acceptable solutions.
5. Implementing the solution. (Try it out.)
6. Evaluating the solution.

## AT THE END, I SAY:

- A good manger does not try to eliminate conflict.
- He tries to keep it from wasting the energies of people.
- If you are having boss and your people fight you openly when they think you are wrong, that's healthy.
- If your people fight each other openly in your presence for what they believe in that's healthy.
- But keep conflict eye ball eye ball …!!!

# CHAPTER 13

# ROLES AND FUNCTIONS IN CONTROLLING

## LEARNING OBJECTIVES

*At the end of this chapter, the student should be able to:*

1. Determine the basic steps of controlling
2. Explain the source and levels of standard
3. List four criteria for setting good standards
4. Discuss the importance of measuring performance
5. Explain the most common causes of problems

## 13.1. WHAT IS CONTROLLING?

Controlling is one of the basic functions of managers. (The other basic functions are planning, organizing, directing). It is the measurement and correlation of performance. Its purpose is to make sure that plans succeed.

## 13.2. WHO CONTROLS?

Control activities are essential parts of the responsibilities of managers at all levels in the organization (first-line managers, department heads, and top management).

## 13.3. BASIC STEPS OF THE CONTROL PROCESS

The basic control process, wherever it is found, and whatever it controls, consist of three steps as follows:

- ♣ setting control **standards.**
- ♣ **measuring** performance against these standards (monitoring).
- ♣ **correcting** deviations from standards.

**Example 1:**

**Controlling blood pressure**

- ♣ Control standard: systolic blood pressure not more than 140 mmHg.
- ♣ Measurement: measurement of BP every month.
- ♣ Corrective action: medication administration

***Relationship between controlling and planning:***

***They are known as the Siamese twins of management.***

## 13.4. SOURCES OF STANDARDS

1) **Official documents.**
2) **Historical analysis of institution's own data.**
3) **Comparative standards (benchmarking**
4) **Scientific.**
5) **Expert opinion:**

## 13.5. LEVELS OF STANDARDS

### 13.5. 1. Minimum standards:

Represent the lowest level of acceptable performance. Appropriate only in health systems and facilities where the general level of care is average so that those below minimum standards are suspected to constitute a significant problem.

Under such situations, the presence of minimum standards is expected to encourage health care institutions to reach and even exceed them with the ultimate result of improving the quality of health services the patients receive.

### 13.5.2. Optimum standards:

The best level of performance under the best possible circumstances. They represent the ultimate goals toward which a health service program would direct its efforts and continuity try to achieve.

They are suitable mainly for health facilities which have achieved a reasonable high level of development concerning their resources, organization and operational systems and processes, and which have well established successful control programs.

### 13.5.3. Empirical / average standards:

Empirical standards are usually averaging that are based on actual practice. They may be based on institutional, regional or national averages within a certain time frame.

Examples of these standards may include:

- The average number of investigations by diagnosis.

- The average hospital stays by diagnosis.
- The caesarean section rates.

## 13.6. CRITERIA FOR SETTING GOOD STANDARDS:

1. Good standards should focus on critical elements, i.e., elements known by their demonstrated relationship to the outcome of service (validity).

♦ **High volume**: affecting large number, e.g., diabetes mellitus.

♦ **High risk:** serious consequences, e.g., pneumonia.

♦ **High cost**: e.g., MRI examination.

♦ **Problem-prone:** problem specific to the facility or department.

2. Current: standards should be reviewed and revised or changed as necessary so that they reflect the changes in medical sciences and technology, as well as changes in the socio-economic condition of the community and in the values and expectations of the people.
3. Good standards should be specific, objective, and preferably expressed in quantitative terms, thus to leading to reliability of measurement.
4. Standards should be reasonably attainable (**realistic**) considering resources and environment:
    - Too high either cause anxiety or tend to be treated with contempt.
    - Too low tend to breed premature satisfaction and complacency.

## 13.7. PARTICIPATION

Participation of all levels involved in the process in setting standards:

♣ Ensures that they are realistic.
♣ Promotes compliance with standards.
♣ Promotes self-control.

## Examples of Hospital Control Standards

**Input standards:**

Hospital care should be provided at a level of 4 hospital beds per 1000 population

**Activity / process standards:**

Except in extreme emergency, all patients undergoing surgery shall have a signed consent.

**Output Standards:**

The postoperative wound infection rate should not exceed 2% of clean surgical cases.

### 13.8. MEASURING PERFORMANCE (MONITORING)

Measuring is the process of assigning a **value** to some objects, events, or phenomena, according to predetermined **rules**.

The assigned value is usually numerical (quantitative), but sometimes it is nominal (qualitative).

The use of the word rules is very critical. It means that the method of assigning value must **explicitly** (spelled out) and **clearly** state what is exactly to be measured, when, and where. This is important for standardization, which means that similar results should be obtained when the same measuring instrument is used by the same person or by different persons.

## 13.9. IMPORTANCE OF MEASURING PERFORMANCE

Provides measures of achievement, so that each member of the organization knows not only his/her performance, but also that of relevant other members.

To provide an objective basis for rewarding members of the organization. To provide data useful in setting new expectations (objectives). For controlling (to identify correctable deviations from the plan).

## 13.10. CHARACTERISTICS OF GOOD MEASURES

**1. Accuracy of collected data**

**2. Reliability of measurement**

**3. Corrective Action**

**4. Performance meets standards (In-control loop):**

- No change is required.
- Continue monitoring.

## 13.11. PERFORMANCE BELOW STANDARDS:

Determine the cause of the deviation.

- If the deviation is **acceptable**, or its **cause is uncontrollable,** activities continue with no intervention **(Acceptance control loop).**

***Examples of acceptance deviation: The size of the waiting list increases because one of the theatres is being renewed.***

- If deviation is unacceptable and the cause is controllable, intervention should occur **(Required change control loop):**

***Examples: Average stay higher than standard due to unacceptable reasons: Physicians do not discharge patients in time.***

## 13.12. ACTUAL PERFORMANCE EXCEEDS STANDARD (POSITIVE CONTROL LOOP):

If **desirable** (e.g., % of pregnant women attending antenatal care clinic exceeding standard in one rural health unit of the district):

- Reinforce.
- Freeze in place.
- Replicate elsewhere (change standard in other units of the district).

If **undesirable**, the cause should be determined and either "acceptance" or "required change" control loops should be used.

*For example, physical plant maintenance expenditures may be below budget, yet this may be undesirable if preventive maintenance is postponed.*

## 13.13. STEPS OF CORRECTIVE ACTION

1. Identify the root cause of the problem.
2. Develop a plan for corrective action that includes:
    - The changes required.
    - The appropriate actions needed to overcome the problem.
    - The person or group responsible for implementing and overseeing the actions (adequate authority).
    - The time frame within which the actions should occur.
    - Implement corrective action.
    - Monitor and evaluate performance.

## 13.14. MOST COMMON CAUSES OF PROBLEMS ARE

- Insufficient knowledge.
- Behavioral problems.
- Defects in systems (usually responsible for around 70% of problems).

## 13.15. APPROPRIATE CORRECTIVE ACTION

Insufficient knowledge may be corrected by:

- ♦ Training and continuing education.
- ♦ Providing reference source, e.g., books, periodicals, internet

## 13.16. BEHAVIOUR OR PERFORMANCE DEFICIENCIES MAY BE CORRECTED BY:

- ♣ Counseling.
- ♣ Emphasizing supervision and motivation.
- ♣ Changing duties.
- ♣ Transferring.
- ♣ Withdrawing certain privileges.
- ♣ Discipline.

## 13.16 DEFECTS IN SYSTEMS MAY BE CORRECTED BY:

- ♣ Changing policies and procedures.
- ♣ Changing lines of authority.
- ♣ Changing span of control.
- ♣ Redistribution of staff.